FACULTY OF HEALTH SCIENCES
DEPARTMENT OF COMMUNITY MEDICINE
CENTRE OF SAMI HEALTH RESEARCH

Ethnic discrimination and bullying in relation to self-reported physical and mental health in Sami settlement areas in Norway

The study

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Foto på forsiden: bildet er tatt i Kanstadbotn, Lødingen Kommune, Nordland i 1897.
Fotograf: ukjent.

Kvinne helt til venstre er *Ingrid Olesdatter Sarri Inga* (født: 1862 i Sarre i Sverige i nærheten av Nikkeluokta). Foran henne står datteren *Inga Nilsen Inga* (født: 1894 i Lunkan, Kanstadfjord, Lødingen). I komsa ligger *Karen Nilsendatter Inga* (født: 1897 i Kanstadfjord).

I midten av bildet står mannen *Nils Andersen Inga* (født: 1853 i Jukkasjavri, Ohkkiras, Sverige). Han var gift med Ingrid. Foran Nils sitter *Ole Nilsen Inga* (født: 1887 i Sortland). Damen til høyre for Nils er *Elen Olesdatter Sarri Svonni* (født: 17. september 1859 i Kanstadfjord, Lødingen). I armene holder hun *Inger-Anna Andersen Svonni* (født: 14. mars 1894 i Trøssemark, den gang Trondenes kommune) (Inger-Anna er disputantens mormor). Sittende på kne helt til høyre i bildet er gutten *Tomas Peder Andersen Svonni* (født: 19. sep 1890 Sortland).

Kilde: Inge Anders Svonni Anderssen

“Ved sammenligning af disse tre nationaliteter vil man finde, at i retning af legemshøide, kropsbygning, kræfter og udholdenhed star nordmændene høiest, dernæst kvænerne; langt underlegne baade i legemlig og aandelig henseende er sjøfinnerne, der i modsætning til de to førstnævnte synes at være i tilbagegang og at fortrænges af de andre, hvis ikke indgiftning med dem finder sted…” (Skjervøy 1900)

”Selv ”Nordmandens” positive egenskaper ville gå til grunne i det avkom han avlet med sjøfinnen: Jeg har i det hele ikke meget godt å si om blandingsbefolkningen – den synes væsentlig at ha arvet begge moderracers slette egenskaper.” (Karlsøy 1910)

Utdrag fra distriktslegenes medisinalrapporter fra

Skjervøy og Karlsøy gjengitt i ”Nordlendingen”

av (Edvardsen, 1997)

“In comparing the three nationalities one will find, in reference to body height, build, strength and endurance the Norwegians are superior, followed by the Kvens; far inferior in both physical and spiritual respects are the sea-Sami, whom in contrast to the former two appear to be in decline and displaced by the others, unless inter-racial marriages with them occur ...” (Skjervøy 1900)

“Even the positive attributes of ‘the Norwegian’ would go to rack and ruin in the offspring he reared with the sea-Sami : I generally have few fond words for the ethnically mixed population — it appears, essentially, to have inherited the bland properties of both mother races.” (Karlsøy, 1910)

Excerpts from the district doctors’ medical reports from Skjervøy and Karlsøy, reproduced in The Northerner *(In Norwegian:* Nordlendingen*) by Edvardsen, 1997*

Figure 1: Sápmi: home of the Sami, stretches over four countries – Norway, Sweden, Finland and Russia.

Source: Gűnter Minnerup, UNSW, Australia.

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Tromsø, October 2010
Ketil Lenert Hansen

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Sammendrag (Abstract in Norwegian)

Vitenskapelig kunnskap om de norske samenes helse og levekår har økt de siste årene, spesielt etter at Senter for samisk helseforskning ble etablert i 2001. Forut for oppstarten av virksomheten ved senteret ligger ulike offentlige dokumenter til grunn. Fra 1995 foreligger *NOU 1995:6 Plan for helse- og sosialtjenester for den samiske befolkningen i Norge*. Dette dokumentet er det første offentlige dokumentet som tok for seg behovet for å få på plass en helse- og sosialtjeneste for den samiske befolkningen. Utredningen satt søkelyset på behovet for mer kunnskap om samenes helse- og levekår og det ble foreslått at det burde igangsettes en forskningsmessig innsats på dette området. Den begrensende vitenskapelige kunnskapen omkring samenes helse og levekår ikke bare i Norge, men også i Norden og Russland, ble sett i kontrast til mengden av detaljert informasjon om helse og levekår som var og er tilgjengelig for urbefolkning populasjoner i det cirkumpolare området.

Forutsetningen for å få i gang forskning på helse og levekår i de samiske områder var en stor og representativ helseundersøkelse. Denne ble gjennomført i årene 2003-4 i samarbeid med Statens Helseundersøkelser (SHUS), nå Nasjonalt Folkehelseinstitutt. Helseundersøkelsen i områder med samisk og norsk bosetting ble kalt SAMINOR (til sammen 24 kommuner i Nord-Norge og Trøndelagsfylkene). Denne studien har vært hovedsatsingen for senteret. Invitasjon ble sendt til cirka 28 000 personer i alderen 30 og 36-79 år. I alt deltok nesten 17 000 personer (61 prosent). Geografisk omfatter undersøkelsen kommuner der 5-10 prosent eller mer av befolkningen i Folketellingen 1970 hadde en eller flere besteforeldre med samisk språk. I tillegg til spørreskjemainformasjon ble det gjennomført noen fysiske målinger og tatt blodprøver som nå oppbevares i en egen biobank.

I mange vestlige samfunn, har etniske minoriteter og urbefolkningsgrupper blitt utsatt for assimilasjon, rasisme, segresjon, etnisk diskriminering og mobbing. Møtet med det vestlige samfunnet har for mange urbefolknings grupper hatt store helsemessige konsekvenser. Den samiske befolkningen ble forsøkt assimilert (gjennom fornorskningsprosessen), noe som har ført til at store deler av den samiske befolkningen (særlig på kysten) mistet sitt samiske språk og identitet. Selv om den historiske fornorskningsprosessen er over, og vi i dag ser en revitalisering av samisk kultur og identitet i mange samiske områder, har for eksempel etnisk diskriminering av samer i liten grad blitt behandlet i relasjon til helse.

Det overordnede målet for denne avhandlingen er å studere prevalensen av selvvurdert etnisk diskriminering og mobbing blant samer, kvener og etniske nordmenn. Videre å studere om det er en hypotetisk link mellom etnisk diskriminering og helse, og hvis så, studere denne assosiasjonen. Selvvurdert helse (SRH) og psykisk stress (HSCL-10) ble benyttet som helse indikatorer. Helt til sist i avhandlingen blir 19 personlige verdier studert og analysert.

En forutsetning for å kunne si noe om den samiske befolkningen ligger i å kunne definere hvem som er samer og hvem som er ikke-samer. Siden etnisitet ikke blir registrert i offentlig register i Norge, har operasjonalisering av etnisitet hatt stor betydning for våre analyser. SAMINOR studien har vært viktig i arbeidet med å klargjøre ulike definisjoner av samisk etnisitet og tilhørighet. Ved å lage flere kategorier av samisk etnisitet har vi sett at ulikheter i den samiske befolkningen og mellom samer og majoritetsbefolkningen trer tydeligere frem med hensyn til etnisk diskriminering, helse og personlige verdier.

Funnene i vår undersøkelse tyder på at en stor andel av samene opplever diskriminering på grunn av sin samiske bakgrunn. Rundt 4 av 10 samisktalende menn og 1 av 3 samisktalende kvinner hadde opplevd å bli diskriminert. Videre er det samer som bor utenfor de definerte samiske språkområdene som rapporterer høyest nivå av diskriminering. På spørsmålet om du har vært utsatt for mobbing generelt, svarer de samiske respondentene at de har blitt dobbelt så ofte utsatt for det i sammenligning med majoritetsbefolkningen. Denne mobbingen har tidligere (inkludert oppveksten til respondentene) i størst grad foregått på skolen og det siste året (et år før undersøkelsens tidspunkt: 2003-04) skjedd i arbeidslivet og i lokal samfunnet. Samer rapporter også at diskriminerende bemerkninger er den mest vanlige formen for mobbing de har opplevd, i tillegg til baksnakking.

I vår studie finner vi at samene rapporterer noe dårligere selvvurdert helsestatus (SRH) sammenlignet med etniske nordmenn. Videre viser funnene våre at etnisk diskriminering er assosiert med dårligere selvvurdert helse. Våre funn foreslår at diskriminering og lavere sosioøkonomisk status bidrar til denne ulikhet i selvvurdert helse mellom samer og befolkningen generelt.

Samiske og kvenske menn rapporterer høyere nivå av psykisk stress enn etniske norske menn, mens blant samiske og ikke-samiske kvinner var det ingen signifikant forskjell i

rapporteringen. Imidlertid rapporterte samisketalende kvinner mindre psykiske problemer som de har søkt hjelp for; enn de andre gruppene av kvinner. Generelt vet vi fra andre studier av urfolk at etnisk diskriminering er sterk assosiert med dårligere mental helse. Vår studie støtter opp om dette og viser at det er en sterk assosiasjon mellom diskriminering og psykisk stress, målt med 'Hopkins Symptoms Check List' (HSCL-10). Imidlertid rapporterte samer som var diskriminert noe lavere stress nivå enn etniske nordmenn.

Siste del av denne avhandlingen omhandler 19 spørsmål om verditilknytning (Appendiks B). Tretten av disse spørsmålene er kun spurt til dem med samisk bakgrunn. Fra de 19 verdiene, rangerer de samiske respondentene følgende fem verdier som viktigst: (i) naturen (ii), bruk av naturen (iii) familietradisjoner, (iv) tradisjonelle samiske næringer og (v) samisk språk. På den andre siden, vektlegger de moderne samisk kunst og Sametinget som minst viktig for dem. Gjennom bruk at faktoranalyse ble fire dimensjoner identifisert: "tradisjonelle samiske verdier", "moderne samiske verdier", "kontakt med naturen" og "opplevelsen av marginalisering". Alle disse fire dimensjonene reflekterer viktige aspekter ved dagens samiske samfunn.

Abstract

Scientific knowledge about the health and living conditions of the Sami people in Norway has increased in recent years, notably after the establishment of the Centre for Sami Health Research in 2001. Prior to the initiation of activities at the Centre, various public documents constitute the primary source of information. In 1995 the *NOU 1995:6 Plan for health- and social services to the Sami population in Norway* (in Norwegian, *Plan for helse- og sosialtjenester for den samiske befolkningen i Norge*) was published. This document was the first public document to address the need to establish health- and social services for the Sami population. The plan focused on the demand for additional knowledge about the health and living conditions of the Sami, and suggestions were made that a research-based effort should be launched in the field. Scientific knowledge regarding Sami health and living conditions in Norway (as well as in Sweden, Finland and Russia) was seen as limited in comparison to the wealth of detailed demographic information on the health and socio-economic conditions of indigenous peoples in, for instance, North America and Greenland.

An extensive and representative health survey was determined to be a precondition for research into health and living conditions in the Sami areas. Such a survey was conducted in 2003-2004 in partnership with the National Health Screening Service (SHUS) (since renamed the Norwegian Institute of Public Health (NIPH)). The health survey in areas containing mixed Sami and Ethnic Norwegian settlements is known as SAMINOR; the study has been the Centre for Sami Health Research's main priority. Invitations were sent to approximately 28,000 between 30 and 36-79 years of age. In total, nearly 17,000 people participated in the survey (61 percent). Geographically, the survey comprised 24 municipalities in which at least 5 percent of the residents reported in the Census of 1970 to have one or more Sami-speaking grandparents (a clear indicator of Sami ethnicity). In addition to information gained from questionnaires some physical measurements and blood samples were obtained; this material is currently stored in a purpose-built biobank.

Ethnic minorities and native peoples have been exposed to assimilation, racism, segregation, ethnic discrimination and oppression in many Western societies. For numerous indigenous populations the encounter with Western nations has included tremendous consequences in terms of heath. In Sami communities the *Norwegianisation process*, by which the Sami were subject to state-sanctioned assimilation policies, has been described as dramatically impacting

the Sami culture; large parts of the Sami population lost their Sami language and identity. Although the historical aspect of the process has been described in detail, in terms of its implications on health the circumstances have not been thoroughly investigated.

The primary objective of this dissertation is to study the prevalence of self-perceived ethnic discrimination and bullying in Sami, Kven and majority-population individuals, and study the hypothetical link between ethnic discrimination and health, and, if so, analyse the association. Self-reported health (SRH) and psychological stress (HSCL-10) are utilised as health indicators. The dissertation also includes the secondary objective of studying and analysing 19 personal values.

Making statements about the Sami population is conditional on the ability to distinguish between Sami and non-Sami individuals. Operationalising ethnicity has been of great importance to our analysis as ethnicity is not recorded in Norwegian public registries. The SAMINOR study has been of significance in the effort to clarify different definitions of Sami ethnicity and affiliation. By creating several categories of Sami ethnicity we have revealed that the differences within the Sami population become more apparent in regards to ethnic discrimination, personal values, and health.

The findings in our study indicate that a large proportion of Sami individuals experience discrimination based on their Sami background; roughly four in 10 men and one in three women in the Sami I category (i.e., participants reporting Sami language proficiency over three generations) have experienced being discriminated against “often” or “sometimes”. Moreover, Sami individuals living outside the defined *Administrative Area of the Sami Language* report the highest levels of discrimination. Compared to the majority population, Sami participants are twice as likely to respond that they have been subject to bullying. Among respondents that reported bullying previously, the most common location was public schools. For those who reported bullying in the past year, the most common locations were at work and in the local community. The Sami respondents, furthermore, report more often (than Kven and ethnic Norwegians) that discriminatory remarks were the most common forms of bullying.

Furthermore, our results show that ethnic discrimination is associated with inferior self-perceived health; Sami participants report somewhat lower health status than Ethnic

Norwegians. We have found that discrimination and lower socio-economic status contributes to the inequality in self-reported health between the Sami and the general population.

Our study finds that Sami and Kven males report higher levels of psychological distress than the general population of males. Interestingly, in women the effect is statistically insignificant. Generally, we know from earlier studies that ethnic discrimination is closely related to poorer mental health in indigenous peoples. Our study supports this conclusion and unveils a strong association between discrimination and psychological distress, as measured using the Hopkins Symptoms Check List (HSCL-10).

The final part of this dissertation deals with 19 questions on the internalisation of values (Appendix B). Thirteen of these questions are only posed to participants of Sami background. From the 19 values the Sami respondents range the following five values as most important, in descending order: (i) Nature (ii), Utilisation of nature (iii) Family traditions, (iv) Traditional Sami industries og (v) Sami language. At the opposite end of the scale, modern Sami art and the *Sameting* (Sami Parliament) are considered the least important values. Through the use of factor analysis, four dimensions were identified: Traditional Sami Values, Modern Sami Values, Contact with Nature and Experience of Marginalisation. These four dimensions reflect important aspects of contemporary Sami society.

Čoahkkáigeassu (Abstrakt in Sami)

Dieđalaš máhttu Norggabeali sámiid dearvvašvuođa ja eallinvuogi birra lea lassanan maŋemus jagiid, erenomážit mannjel go Sámi dearvvašvuođadutkama guovddáš ásahuvvui 2001:s. Almmolaš čállosat leat vuođđun álggaheapmái. 1995:s lea NOU 1995:6 *Plan for helse- og sosialtjenester for den samiske befolkningen i Norge.* Dát čálus lea vuosttaš almmolaš dokumeanta mii váldá ovdan dárbbu oažžut dearvvašvuođa ja sosialabálvalusa sámi álbmogii. Čielggadeapmi čájehii dárbbuid eanet máhttui sámiid dearvvašvuođa ja eallinvuogi birra, ja evttohuvvui ahte álggahuvvo dutkan dán birra. Ráddjejuvvon dieđalaš máhttu sámiid dearvvašvuođa ja eallindiliid birra, ii dušše Norggas, muhto maid Ruotas, Suomas ja Ruoššas, veardádallojuvvui bienalaš diehtohivvodagain mii gávdno dearvvašvuođalaš , demográfalaš sosioekonomalaš diliid birra álgoálbmogiid birra omd Ruonáeatnamis, Davvi-Amerihkás ja Australias.

Eaktun álggahit dutkama dearvvašvuođa ja eallinvugiid birra sámi guovllus lei stuorra ja dárkilis dearvvašvuođa dutkamuš. Dutkamuš čađahuvvui jagiid 2003/04 ja lei ovddasbargu Stáhta dearvvašvuođaguorahallamiin (SHUS), dálá Álbmotdearvvašvuođainstituhtain . Guovlluin sihke sámi ja dáža suovain gohčoduvvui SAMINOR dearvvašvuođadutkamuššan (oktiibuot 24 suohkana/gieldda Davvi-Norggas ja Troandinfylkkain). Dat dutkamuš lea guovddáža váldovuoruheapmi. Bovdehus sáddejuvvui sullii 28 000 olbmuide 30 ja 36-79 jagiin. Oktiibuot oassálaste 17 000 olbmo (61%). Geográfalaččat siskkilda iskos suohkaniid/gielddaid gos 5-10% vai eanet olbmuin atne okta vai eanet áhku ja/vai ádjá geat sámástit. Lassin jearadanskolvidieđuide, ledje maid fysalaš mihttosat ja varrageahččaleamit mat dál vurkojuvvojit sierrá biovuorkkás

Ollu oarji servodagain leat etnalaš unnitloguálbmogat ja eamiálbmotjoavkkut vásihan assimilašuvnna, rasisma, segrešuvnna, etnalaš vealahallama ja givssideami. Deaivvadeapmi oarjeservodagain lea mielddisbuktán stuorra dearvvašvuođalaš čuozahusa ollu eamiálbmotjoavkkuide. Sámi álbmot geahččaluvvui assimilerejuvvot (dáruiduhttimiin), dát dagahii ahte stuorra oassi sámi álbmogis (erenomážit rittuguovlluin) massii sámegiela ja sámi identitehta. Vaikko historjjálaš dáruiduhttinproseassa lea nohkan, ja mii otne oaidnit ahte sámi kultuvra ja identitehta ovdánahttá máŋga sámi guovlluin, lea ovdamearkka dihte etnalaš vealahallan sápmelaččain unnán meannuduvvon dearvvašvuođa oktavuođas.

Dán barggu váldoulbmil lea iskat prevaleanssa etnalaš vealahallama ja givssideami sámiid, kvenaid ja etnalaš dážaid gaskkas sin iežaset vásáhusaid vuođul. Viidásut vel iskat jus lea hypotehtalaš golus gaskkal etnalaš vealaheami ja dearvvašvuođa, jus lea, de iskat dan assosiašuvnna. Dán oktavuođas lea iešveardiduvvon dearvvašvuohta (SRH) ja psykalaš hušša (HSCL-10) geavahuvvon dearvvašvuođaindikáhtorin. Barggu loahpas iskojuvvojit ja analyserejuvvojit 19 persuvnnalaš arvvut.

Eaktun sáhttit dadjat juoga sámi álbmoga birra lea máhttit rádjet geat leat sápmelaččat ja geat eai leat. Go etnisitehta ii logahallo almmolaš logahallamis Norggas, de lea etnisitehta operašonaliseren leamaš dehálaš min analysaide. SAMINOR iskan lea leamaš dehálaš bargguin gávnnadit iešguđetlágan definišuvnnaid sámi etnisitehtas ja gullevašvuođas. Go dahká eanet lágiid sámi etnisitehtain, de oaidná ahte sámi álbmoga iešguđetláganvuođat šaddet čielgaseappot go geahččá etnalaš vealahallama, persovnnalaš arvvuid ja dearvvašvuođa.

Bohtosat min iskosis čájehit ahte stuorra oassi sápmelaččain vásiha vealahallama sin sámi duogáža dihte. 4:s 10 almmáiolbmuin ja 1:s 3 nissoniin geain lei sámegielat máhttu 3 buolvvain (Sami 1), ledje vásihan vealahallama. Viidásut leat sápmelaččat geat orrot ráddjejuvvon sámi giellaguovllu olggobealde, geat raporterejit vealahallama alimus ceahkis. Jearaldagas jus leat vásihan givssideami jorbadit, vástidit sámi respondeanttat ahte sii leat duppalit nu dávjá givssiduvvon go buohtastahttá majoritehtaálbmogiin. Givssideapmi lea ovdal (respondenttaid bajásšaddanáigi lea mielde) dáhpáhuvvan skuvllas ja maŋemus jagi (jagi ovdal iskosa: 2003-04) dáhpáhuvvan bargooktavuođas ja báikkálaš servodagas. Sápmelaččat maid raporterejit ahte vealahallan-cuigomušat leat dábáleamos givssidanvuohki maid sii leat vásihan, lassin bahádallamii.

Min iskosis mii oaidnit ahte sápmelaččat raporterejit veahá heittohut iešveardiduvvon dearvvašvuođadilli (SRH) buohtastahtton etnalaš dážain. Viidásut čájehit min bohtosat ahte etnalaš vealahallan lea assosierejuvvon heittogis iešveardiduvvon dearvvašvuođain. Min bohtosat árvalit ahte vealahallan ja vuolit sosioekonomalaš dilli váikkuha iežálágánvuhtii iešveardiduvvon dearvvašvuođas gaskkal sápmelaččaid ja álbmoga jorbadit.

Sámi ja kvena almmáiolbmot raporterejit alibui cehkiid psykalaš huša go etnalaš dáža almmáiolbmot, muhto sámi ja ii-sámi nissoniin ii leat signifikánta erohus raporteremis.

Liikká raporterejit sámi nissonat (geain lei sámegielat máhttu 3 buolvvas (Sami 1)), unnit psykalaš váttisvuođaid masa sii ledje ohcan veahki; go nissonat eará joavkkuin. Jorbadit mii diehtit eará álgoálbmuid iskosiin ahte etnalaš vealahallan lea assosierejuvvon heittogis mentála dearvvašvuođain. Min iskos doarju dán ja čájeha gievrra assosiašuvnna gaskkal vealahallama ja psykalaš huša, mihtiduvvon "Hopkins Symptoms Check List" (HSCL-10) mielde. Liikká raporterejedje sámit geat ledje vealahuvvon unnit hušša-ceahki go etnalaš dážat, geat maid ledje vásihan vealahallama.

Dan guorahallama maŋimus oasis leat 19 jearaldaga arvogullevašvuođa birra (Appendiks B). 13 dain jearaldagain leat jerron dušše olbmuin geain lea sámi duogáš. Daid 19 arvvuin, sámi respondeanttat árvvoštallet čuovvovaš vihtta arvvu deháleamosin: (i) luondu (ii), luonddugeavaheapmi (iii) bearašárbevierru, (iv) árbevirolaš sámi ealáhusat ja (v) sámegiella. Nuppi bealde, de árvvoštallet ođđaáigásaš sámi dáidaga ja Sámedikki unnimus dehálažžan alcceseaset. Oasseanalysa geavaheamis njeallje oli identifiserejuvvoje: "árbevirolaš sámi arvvut", "ođđaáigásaš sámi arvvut", "luondduoktavuohta" ja birzziidvásáhusat. Visot dát njeallje oli reflekterejit dehálaš geahččanguovllu otná sámi servodaga dáfus.

LIST OF PAPERS

I: Hansen KL, Melhus M, Høgmo A, Lund E. Ethnic discrimination and bullying in the Sami and non-Sami populations in Norway: The SAMINOR study. *International Journal of Circumpolar Health 2008* Feb;67(1):97-113.

II: Hansen KL, Melhus M, Lund E. Ethnicity, self-reported health, discrimination and socioeconomic status: a study of Sami and non-Sami Norwegian populations. *International Journal of Circumpolar Health 2010*

III: Hansen KL, Sørlie T. Ethnicity, ethnic discrimination and psychological distress: a study of Sami and non-Sami populations in Norway. (Accepted by Transcultural Psychiatry. McGill, Canada 21-Jul-2010)

IV: Hansen KL, Høgmo A, Melhus M, Lund E. Sami value patterns (Submitted).

The papers will be referred to in the text by the Roman numerals.

INTRODUCTION

Background

The basis for any effort to combat discrimination and ensure equal living conditions among ethnic groups starts with respecting human rights and acknowledging that these legal rights apply to all human beings. However, human rights must extend to a real-world situation in which everyone is able to participate in society and utilise available resources (Ministry of foreign affairs, 1999). As the indigenous people of Norway, the Sami minority have a right to extraordinary protection to preserve their culture. The Norwegian Government is responsible for facilitating the security and development of the Sami language, culture and social welfare (Ministry of Local Government and Regional Development, 2002). The Racism and Discrimination Act (2006 Amendment) describes measures to fight discrimination of the Sami people or Sami individuals. The Act emphasises that an effective anti-racism and anti-discrimination campaign requires a continued, focused and long-term approach. One of the objectives in this effort is to acquire up-to-date research on ethnic discrimination in order to develop efficient measures to tackle discrimination (Ministry of Local Government and Regional Development, 2002). The Act is further supported by Government reports (Ministry of Health and Care Service, 2007; Ministry of Labour and Social Inclusion, 2008) that highlight the importance of a strong focus on research-based insight into discrimination of the Sami populace.

The Sami population

The Sami are the natives of Scandinavia and they live in the northern regions of Fennoscandia in what today comprises the northern areas of Norway, Sweden, Finland and Russia's Kola Peninsula (Figure 2). The Norwegian government has ratified the Sami as the indigenous people in Norway (ILO-convention no 169, 1990). The Sami include several subgroups stratified by different geographical areas and dialects (Jernsletten, 1993). The size of the Sami population has been reckoned to approximately 70,000-100,000, but estimates vary accordance with criteria used like genetic heritage, mother tongue and the personal sense of ethnicity. The largest proportion of Sami is believed to reside in Norway (60,000), followed by Sweden (36,000) and Finland (10,000), with the lowest proportion residing on the Russian Kola Peninsula (2,000) (Statistics Norway, 2010). Moreover, it is difficult to operate with

some minimum- or maximum numbers due to the fact that there are no current demographic numbers to indicate the size of the Sami population, due to a lack of information on ethnicity in public registers. In Norway, about one-third of the Sami live in Finnmark county (Spein, 2007) . Current figures from 2010 found that 13,890 Sami were recorded in the Norwegian Sami electoral register (Statistics Norway, 2010).

Figure 2. Sápmi, home of the Sami, stretches over four countries – Norway, Sweden, Finland and Russia.

Source: Nordic Sami Institute

Sami are engaged in a variety of livelihoods, including farming, fishing, trapping, sheep and reindeer breeding and herding. Although consider as 'traditional' and a cultural marker of the

Sami, reindeer herding was of relatively recent vintage, developing during the sixteenth century. In Norway and Sweden, but not in Finland; semi-nomadic reindeer herding is, by law, an occupation strictly reserved for Sami (Young & Bjerregaard, 2008). Data from 2009 found that only a minority (3,010 individuals) of the Sami in Norway is occupied in reindeer herding with slightly more men than women (Statistics Norway, 2010). And today, many Sami live in the large cities, especially Alta, Tromsø and Oslo, and are involved in all the modern professions, occupations, and trades (Young & Bjerregaard, 2008).

Today the challenge faced by the Sami population consists of conserving traditional knowledge, values and culture traits while both the local community and the world continues to change (Flemmen & Kramvig, 2008) Many Sami people find themselves in a transitional state where it is important to adapt to a new world without losing sight of the values of the traditional world (Young & Bjerregaard, 2008).

The Sami language

The Sami language belongs to the Finno-Ugric branch of the Uralic language family. The closest linguistic neighbours of the Sami are the Finns, The Karelians and the Estonians (Hassler, 2005). There are ten different Sami dialects or languages, and the 'borders' between them cross nation-state boundaries. In Norway approximately 25,000 Sami individuals are proficient in the Sami language, of which about half can speak, read and write Sami whereas for the other half the language is primarily a spoken language only. It is difficult to provide an exact distribution of the different dialects, however Northern Sami is clearly the most common of the Sami languages in Norway (Ministry of Labour and Social Inclusion (now: Ministry of Labour), 2009).The Sami language has had and has a natural role in the traditional Sami community, as an integrated part of the Sami social and cultural life. Today the Sami inhabitants of Norway possess distinct linguistic traits, depending of where they are living (Ministry of Labour, 2009). Further, the use of the Sami language in Norway can be divided into three main areas: the inner Finnmark (Kautokeino, Karasjok, Nesseby and Tana), the coastal area (Rest of Finnmark, Troms and Nordland; north of Saltfjellet) and the southern area (Nordland; south of Saltfjellet and Nord- and Sør-Trøndelag). In the inner Finnmark Sami language is in everyday use, 66.3% of the participants reported use of Sami as language at home and the number for the grandparents and the parents is about the same, between 68.0-

73.4%, see table 1. The Sami in the coastal area has traditionally been Sami-speaking, but because of the assimilation process there has been a change in language, from Sami to Norwegian, over the last three-generation (Høgmo, 1986). Table 1 shows the linguistic assimilation; among 17-20 % of the grandparents had Sami language at home[1], for the parents 14% and only 7% for the participants. In the southern area the Sami population live in small, scattered clusters and families, and reindeer herding have been a main livelihood (Jernsletten ibid.) The number of Sami in this region has remained constant for several generations and the Sami language has also survived because of the reindeer herding. Table 1 show this; the use of Sami language for the grandparent's generation was among 4.3-4.8%, for the parents 3.7-4.0% and the participants 3.4%.

Table 1. Sami language used/use at home for grandparents, parents and participants divided into three main areas (Numbers in percent)

	Grandparents				Parents		Participants
	Father's father	Father's mother	Mother's father	Mother's Mother	Father	Mother	
Inner Finnmark	68.0	70.0	71.7	73.4	68.7	71.5	66.3
Coastal area	16.8	18.1	19.1	19.7	14.0	14.6	7.0
Southern area	4.8	4.8	4.4	4.3	4.0	3.7	3.4

Source: The SAMINOR study

The Kven population

The Kvens are a people that emigrated form the northern parts of Finland and Sweden to northern Norway in the eighteenth and nineteenth centuries due to povery and famine in their native countries. Kvens speak their own language which is an old Finnish language. In 1996 the Kvens were granted minority status in Norway, and in 2005 the Kven language was recognised as a minority language in Norway (Hyltenstam, 2003). Kven and Sami people share a common history of strong linguistic and cultural assimilation (Jernsletten, 1993). However, the SAMINOR study was designed to study the Sami population and did not include the main settlements of the Kven population (Lund et al., 2007).

[1] Reported by the participants in the SAMINOR study.

Colonisation of the Norwegian Sami

To understand and counteract discrimination of the Sami, and to minimise social inequality, it is crucial to be aware of how such discrimination manifested itself in the recorded history of Sami interaction. This section takes a closer look at how the Norwegianisation process affected the daily life of the Sami. The purpose of the following analysis is *not* to offer a complete picture of Sami history, *but to reveal important political and social conditions* that greatly influenced, and still influences, Sami society.

The Sami have a long, continuous historical relationship to the High North (including parts of contemporary Norway, Sweden, Finland and Russia). In this area, the Sami people developed viable trades as well as (multiple) distinct languages, culture and identity. This time extends back to before the area was colonised and before the formation of nation-states on partially Sami soil. Until the 1970s, Sami history was commonly deemed not to belong to the field of historical disciplines (Hansen, Minde, & Olsen, 2004). Thus, the Sami were considered 'a people without a history' and such attitudes were visibly expressed when the national histories of the Nordic countries were to be written. In historical accounts the reason for neglecting the Sami people seems to be that the notion of 'settlement' was made synonymous with the permanent farming settlements represented by Norse peoples. The Sami adhered to a nomadic lifestyle and their living areas were often depicted on maps as 'uninhabited'. Only when the Sami presence was used to explain circumstances relating to the Norse societies were they mentioned in historical accounts. In Said's words, "They were present, but ignored beyond their usefulness as part of the setting" (Hansen et al., 2004). In the nineteenth century, however, the origin of the Sami people was put on the scientific agenda, partly due to European social and scientific trends such as the growth of nationalism and the emergence of the theory of evolution. The nation-state ideal of consistent political, cultural and ethnic boundaries would affect how the Sami were regarded, and inevitably influenced Sami living conditions. The view on Sami 'origin'—or historical identity in Europe's north—would be considerably altered during the second half of the nineteenth century. Before this time, researchers and scientists commonly believed that the Sami descended from the populations inhabiting Scandinavia and northern Europe during the Stone Age. This perception, however, was soon dismissed and the Sami were ascribed the more limited status of the indigenous population of northern Fennoscandia. In the early twentieth century this status was further marginalised. Several scientists began questioning whether the Sami had a genuine 'indigenous' presence in the Nordic region. The Sami had migrated to the Nordic region from

the East, it was argued, long after the current majority population had found its place there (Hansen et al., 2004). The reason for this remarkable change was associated with several circumstances which are collectively referred to as the Norwegianisation process. The Norwegianisation process alludes to the Norwegian Government's Sami policy, which was publicly initiated in the mid-19th century and abolished in 1959 (Jensen, 2005) with the aim of acculturating the Sami minority into the Norwegian culture. In 'official discourse' the Sami were described as inferior, uncultured and partially without material rights. In plain language, the goal was to assimilate the Sami and to force upon them a language change (Jernsletten, 1993). The dominant perception was that the Sami were on a 'low' cultural level. This policy was inspired by nationalism, social Darwinism and national security rationales, and its goal was cultural disintegration of Sami society.

The fact that Norway was established as an independent state in 1814 did not immediately lead to a negative view of Sami language and culture. However, from the mid-19th century, national romanticism flourished in Europe and spread to Norway, where the 'love of one's country' was primarily expressed through idolising the rural farming lifestyle as idyllic (*bondeidyll*). As new technologies such as the mass media (including newspapers and telephone networks) brought the nation's regions closer together, national romanticism remained prominent, and as literacy levels rose steadily, the people were inundated with the Norwegian *bondeidyll*. Thus, the Sami were restricted from learning their own language and the Sami cultural heritage became worthless in Norwegian eyes (Jensen, 1991).

Towards the end of the nineteenth century cultural researchers applied Darwin's tenet—only the fittest survive—to the increasing rivalry in human social development, thereby founding what we call *Social Darwinism* today. The essence of Social Darwinism may be summarised as a way to arrange groups of people according to the level of development the peoples of the world are considered to have attained. According to this model, the most 'primitive' peoples are placed last and the most 'civilised' or 'urbanised' first, causing the Sami and Kven to be regarded as lagging behind the Ethnic Norwegian population in terms of development. A consensus among leading figures in society was thus established: these people had to be 'Norwegianised' or face extinction (Jensen, 1991).

In relation to national security policy, the government was worried about the potential for Finnish or Russian expansion into northern Norway. The fear of Finland and Russia was

central to the wide-ranging Norwegianisation measures that were implemented; it was crucial that Sami and Kven people felt closely associated with Norway (Norwegian nationalism). Einar Niemi and Knut Eriksen (1981) coined the phrase "The Finnish Threat" (*den finske fare*) for what the Norwegian authorities officially decreed in the year 1860. Finnish nationalism was growing strongly around this time, adding to the urgency of imposing Norwegian nationalism onto Finnish immigrants to northern Norway (designated as *Kven*) (Jensen, 1991).

Revitalisation of Sami culture and identity

The Sami society has undergone an ethnic and cultural revival over the last decades (Hansen, Melhus, Høgmo, & Lund, 2008; Pedersen & Høgmo, 2004), and the modern Sami history have been a fight for recognition as human beings, culture and as a indigenous people. The Alta Dispute was an event of great significance in raising the Sami conscience in the years 1979-1981 when the Sami activist organized themselves to oppose the plans for damming the Alta river. The Sami people were strongly supported both nationally and internationally. This dispute was followed by a period of committee reports, and the results began to show in the late 1980s in the form of important changes in legislation and building of modern Sami institutions, as the Sami Parliament, which gave the Sami people a strong feeling of belonging to a larger global community and has strengthened their position as an indigenous minority in relation to the nation state (1990). These changes were formally recognised in the Norwegian Constitution (§110a), which requires the Government to facilitate the development of the Sami language, culture and social welfare (Eidheim, 2000; 1990). So today, the Sami population has achieved more cultural equality and is less socially disadvantaged compared other First Nation people; this is shown in fields of education, health, research, arts, festivals and politics. For example, due we see a growing well-educated younger population among the Sami (Stordahl, 1996).

The Sami population still influenced by past assimilation policies

Although policies of assimilation may be a thing of the past in terms of Norwegian society and politics, the negative consequences project into the present and, indeed, into the future. It takes time to fundamentally change general frameworks in Norwegian politics, legislation and ordinances, as well as myths and attitudes, to appropriately address Sami culture, language, traditions and social needs (health care, education and employment). Many people remain influenced by past assimilation policies despite the official legislation having been reversed (Ministry of Labour and Social Inclusion, 2008). Simply "being different" is often the source of discrimination and harassment, and, as a minority population, the Samis are vulnerable (Høgmo, 1998). Studies conducted abroad reveal ethnic discrimination to be closely associated with health issues, pertaining to self-reported, general health statements as well as self-reported mental illness. The most evident association is that to mental illness (Ahmed, Mohammed, & Williams, 2007; David R Williams, Haraold W Neighbors, & James S Jackson, 2003; Harris et al., 2006; Harris et al., 2006) Thus, it is important to focus on questions relating to discrimination and bullying of Samis from a health perspective in order to even out social differences in terms of physical and psychological well-being (Ahmed et al., 2007; Harris et al., 2006; Harris et al., 2006; Ministry of Health and Care Service, 2007).

The notion of racism

As with any discussion of ethnic discrimination we can hardly avoid mentioning racism as a notion, and discuss the debate surrounding racism (Høgmo, 1998). However, what is really meant by words such as race, racism, ethnic discrimination, nation, integration and culture? Many people use these words in everyday life, intending a certain meaning; however, when asked, most fail to recognise the true meaning of these words. Researcher Torgeir Skorgen has observed that racism is an understudied field in Norway - despite the fact that the debate on racism is notable in both scope and intensity. Events such as the so-called *Ali Farah scandal* (in which a Norwegian-Somali man was refused medical attention in the Sofienberg public park) exemplifies racist tendencies (Skorgen, 2009).

Racism is essentially an umbrella term for ideas about 'us' and 'them'. As an idea, racism is the predominant doctrine that divides humanity into different categories, or races, in which some are superior to others. In the nineteenth century scientists believed people from different parts of the world belonged to different human races. These human races, it was thought, had dissimilar inborn qualities which could only belong to the respective races. Theories of race attempted to combine and relate complex ideal or typical biological attributes (such as 'facial angle', skull shape, etc.) to inner mental capabilities. Race, thus, is never objective, but socially created through selection, interpretation and definition (Skorgen, 2002).

Racism did not emerge with the first immigrants to Norway. The notion of 'us and them' has existed for thousands of years. People with darker complexions have been considered to be of lesser value (to whites); 'coloured' people were seen to constitute either a problem, threat or both. From the fifteenth century onwards, European nations conquered other parts of the world, proclaiming the areas to be colonies, extracted valuable goods, and governed the territories and the people living there as they saw fit. Partially, such exploits contributed to the sentiment that Europeans were 'better' than other human beings. During the slave trade, Europeans removed Africans from their homes and transported them by sea to America, where they were forced to work in conditions normally reserved for animals. Norwegian nationals were limited in their contact with other peoples; seafarers were the predominant source of contact through their tales of overseas experiences. As missionaries and sailors described faraway societies, Norway's indirect participation in the slave trade and colonialism contributed further to the knowledge of other peoples (Skorgen, 2004).

Immigration to Norway has existed for centuries. The Hanseatic League (Germans) had a considerable presence in Bergen (1400 - 1760), Finnish labourers in the seventeenth and eighteenth century and a considerable number of Swedes have helped populate the country. Gypsies, Romanis and the Sami, however, were the *visible minorities* and were thus exposed to extensive discrimination and persecution (Høgmo, 1998; Skorgen, 2002).

Racism against the Sami

Norwegian racism extends far back into history. Svein Lund notes that while the slave trade was going on in Africa, northern Scandinavia was also colonised (Lund, 2008). The Sami were seen as subservient; they were wild hedens; and their land was rich in resources. The colonisation took many forms, such as taxation, trade, christening, acquisition of land for agriculture and, as mentioned, Norwegianisation. The practicalities of colonisation and racist ideologies "walked hand in hand", figuratively speaking. In 1776 the *Governor of the North* (*Amtmannen of Nordland*) said:

"These pointless and harmfully running-around Sami are just that which one wishes to have cleared from the country ... This chasing away cannot be done in any more convenient way than have their turf huts and sod houses torn down and burned" (in Lund, 2008).

Even in Europe the myth of the wild nature of the Sami was widespread. In his dissertation on how the Sami were portrayed in French novels and scientific literature in the eighteenth century, Martin Wåhlberg focused particularly on the depiction of Sami sexuality (Landsverk & wåhlberg, 2008). In a novel by Marquis de Sade, the following statement was highlighted: "It is an honour, amongst the Sami, to prostitute their wives to strangers". These declarations were repeated by authors Voltaire and Regnard. The source of this myth was traced to the professor Johannes Scheffer at the University of Uppsala, whom wrote the first dissertation on the Sami: *Lapponia*. In his work Scheffer wrote that there *may have been cases in history* in which the Sami offered their spouses to others. The myth regarding Sami sexual morals was then connected to the race theories of the eighteenth century. Amongst others, Georges-Lous Leclerc, the Count of Buffon, wrote in his *Historie naturelle, générale et particuliére* that the 'despicable sexual morals' of the Sami separated them from other races, and he thus placed them lowest of all human races (wåhlberg, 2009; Wåhlberg, 2008).

Measurements of Sami skulls

In the mid-1850s a novel branch of science — physical anthropology — reached Scandinavia. Through the identification of 'typical' Sami and Nordic racial traits, primarily the shape of the skull, it would be possible to empirically determine and trace which race first inhabited Europe's far north. A number of physical characteristics were associated with the

measurement of skulls. The partitioning doubled as an 'evolutionary scale' and the theories predicted the blonde "long-skulls" (the Nordic race) to be the superior product of evolution both in the bodily and spiritual sense. The Sami, on the other hand, belonged to the "short-skulls" and were described by the researcher Halvdan Bryn as being of a lesser and lower race that did not have a future. He writes: "despite having lived in the immediate vicinity of more highly cultured races, they [the Sami] never arrived at any form of higher culture" (Bryn, 1925). Some of the information was collected from living individuals; other measurements were conducted on skeletons from Christian and pre-Christian burial sites. Often, such excavations were performed in a manner which the Sami considered highly offensive and degrading (Schanche, 2000).

The cause of scientific interest in crania, and, in particular, those of Sami and other indigenous peoples, was closely connected to colonialism, nationalism, and the need for legitimisation of new forms of power exertion through a new world order (Hansen et al., 2004).

Racism and health

"Racism leads to poorer health," says Professor of Social Medicine Per Fugelli. Feeling safe and dignified is decisive to one's health, and constant discrimination removes this sensation from the individual. Further, xenophobia (such as fear of other ethnicities) is not healthy, either; there are no beneficial effects from imagining that we are surrounded by 'bad foreigners'. At any rate, human beings have a need to do the right or 'good' thing, and that may not be accomplished by talking disrespectfully about other people, claims Fugelli (Fugelli, 2000).

How discrimination may affect health

The model disclosed in Figure (figure 3) illustrates three paths through which perceived discrimination may influence health. First, discrimination may have a direct impact on health (Path a). Second, the relationship between discrimination and health may be mediated through exposure from discriminatory events to stress may rise to negative emotion (Path b). If an individual perceives discrimination on a regular basis, these stress responses are typically

activated, potentially leading to a consistently negative emotional state. Chronic, heightened physiological stress responses, such as cardiovascular reactivity and cortisol responses are also included in this pathway. Thus, experiencing discrimination may contribute to health problems via Path c through allostatic load developed by a heightened stress response and negative emotional states. Finally, another mediating path exists from health risk behaviours (tobacco use and alcohol abuse) that may emerge as possible coping mechanisms when discrimination is experienced. As represented by Path e, these activities can have detrimental effects on physical health and contribute to increased risk of multiple major disease outcomes (Pascoe & Smart, 2009).

Figure 3: From Pascoe, EA et al. Perceived Discrimination and Health: A Meta-Analytic Review. *Psychol.Bull., 135,page* 532.

There are also several factors that may moderate the link between discrimination and health. There moderators include social support, coping style, ethnic identity, and personality variables. Having a strong connection to a certain group identity, such as those based on

ethnicity or gender group, may buffer the stress of discrimination by preventing stereotypes from infecting the self-concept (Pascoe & Smart, 2009). For example, Yip, Gee and Takeuchi (2008) found that ethnic identity buffered the association between discrimination and mental health for U.S.-born individuals 41 to 50 years of age.

Present-day racism and ethnic discrimination — cultural fundamentalism

While 'classical' racism has been concerned with external racial characteristics—such as shape of the skull, facial angle, hair and skin—recent attention has focused on ethnicity and identity. Cultural differences are underlined; no mention is made as to which culture is better than the other. Instead, the emphasis is on cultural incompatibility (Barker, 1981; Gullestad, 2006). This supposed incompatibility is the premise for the argument that groups belonging to highly dissimilar cultures should preferably live separately. If different cultures are to live together, there are demands that minorities assimilate towards the majority culture in the social and lingual sense. This may be used to argue for a more restrictive immigration policy. Martin Barker calls it "neo-racism"; in the United States the term 'symbolic racism' is often used. Others, again, consider expressions such as 'cultural fundamentalism' and compares neo-racism to nationalism (Gullestad, 2002). This new form of racism or discrimination is difficult to observe directly; cultural it may be recognised as social exclusion through the placement of discriminated groups in lower socio-economic strata.

In many ways one may speak of ethnic discrimination as primarily concerning 'mine' and 'your' identity, and as being about how we perceive 'ourselves' and 'the others' as people . "These days many racial stereotypes seem to be sailing with false flags; that is, under the guise of newer notions such as ethnicity" (Skorgen, 2002). The word *ethnicity* comes from Greek, meaning *people*. There are several associations that are used within the social sciences tied to the notion of ethnicity. Often, the expression describes people whom are 'different' to ourselves in terms of language, clothing, way of life, behaviour, et cetera. (Thuen, 1995). According to this starting point, one thus attempts to separate different ethnic groups by alluding to external and stabile traits of the subjects. Barth (Barth, 1982) describes that by, for example, using 'ethnicity' to refer to a group of people different to one's own, we contribute to our mental organisation of our experiences regarding ourselves and the world.

Ethnicity, therefore, becomes an epithet for describing differences between people, so that, as described by Barth, the experience of ourselves and the surrounding environment is categorised in terms of 'us and them'. This intersection of different people can occur in various ways; the different ethnic groups may live together in consensus or the encounter between them may involve conflict. Because ethnicity is mentioned in the interaction between different groups, we may state that the issue of ethnicity has an *interethnic quality*. This means that ascription of ethnicity will be a result of an interaction situation where the cultural aspect of ethnicity is brought to the forefront (Barth, 1982).

Further, the notion of ethnicity is tainted by subjective delusions about cultural limits, whether they be based on language, religion or 'race'.

Previous research on discrimination among the Sami

A significant amount of qualitative social sciences research is available, and describes discrimination of Samis and their experiences under assimilation policies, particularly as originating in the educational research tradition, which focuses on the public education system (Eidheim, 1977; Høgmo, 1998). However, there is a lack of quantitative data to establish the prevalence of ethnic discrimination of Samis in Norway. Only one study on self-reported discrimination among Samis on Norway may be found in the literature. This study included 545 respondents and revealed that about one in four said they had experienced discrimination based on the fact that they were Sami (Josefsen, 2006). A comparable study from 1998 of Samis in Sweden showed that one in three Samis were exposed to ethnic discrimination (Lange, 1998). Our study contributes to the body of knowledge on self-reported discrimination and harassment of Samis based on a relatively large number of Sami inhabiting important Sami municipalities and districts in Norway.

AIMS OF THE THESIS

Based on historical circumstances surrounding now-abandoned assimilation policies, in which the majority culture threatened Sami culture and language, this study aims to investigate the occurrence of self-reported Sami ethnic discrimination, bullying, health and value patterns in the multicultural Norway of today. Such evidence will be contrasted with recent social developments, in which we have witnessed a strong, positive change in attitudes toward Sami culture, language and identity.

The following research aims were investigated:

1. To investigate the prevalence of self-reported experiences of ethnic discrimination and bullying among 36 to 79 years old Sami and non-Sami adults (Paper I);

2. Examine the association between ethnicity, social factors and self-reported health conditions (Paper II);

3. To identify the prevalence of psychological distress and to investigate the associations between discrimination and psychological distress (Paper III);

4. To study and contrast personal values in this multi ethnic population (Paper IV).

METHODS AND SUBJECTS

Study design

This thesis is based on questionnaire data from the population-based study of health and living conditions in areas with mixed Sami, Kven and Norwegian majority population (the SAMINOR study), for which data were collected during 2003 and 2004. The SAMINOR study is a cross-sectional epidemiological study of adults in the five northernmost counties of Norway: Finnmark, Troms, Nordland, Nord-Trøndelag and Sør-Trøndelag. The respondents completed three questionnaires covering demographic, socioeconomic, ethnicity, experiences of ethnic discrimination/bullying, current health situation and other social characteristics. Further details on the collection process and methods have been published previously by Lund *et al*. (Lund et al., 2007).

The SAMINOR study was the responsibility of the Centre for Sami Health Research, Institute of Community Medicine at the University of Tromsø, in collaboration with the National Screening Program for Cardiovascular Diseases, SHUS, now incorporated into the National Institute of Public Health (Lund et al., 2007).

Procedure

The SAMINOR study intended to include the populations of municipalities in Norway (Figure 4) in which more than five percent of the population reported themselves as Sami in the 1970 Census (Aubert, 1978). In the national census of 1970 a survey of Sami ancestry was performed in preselected census tracts in the three northernmost counties of Norway: Nordland, Troms and Finnmark (Appendix A). The census was carried out by Statistics Norway in cooperation with Sami organisations. Information on ethnicity in the 1970 Census represents the most up-to-date source of Sami ethnicity and identity in the North Norway and remains the only source of acceptable quality to be used in research. In addition, some selected districts were selected from municipalities with an overall lower proportion of subjects with Sami ethnicity.

Figure 4. Study areas of the SAMINOR study

Study populations

In total 28,071 people were drawn from the Central Population Register in the defined SAMINOR area, 27,987 were eligible to participate (Figure 5), and a total of 16,968 (60.6%) *did* participate. Residents participating in the 2003 selection were eligible if born between 1925-1967 and in 1973. Those participating in the survey in 2004 were eligible if born

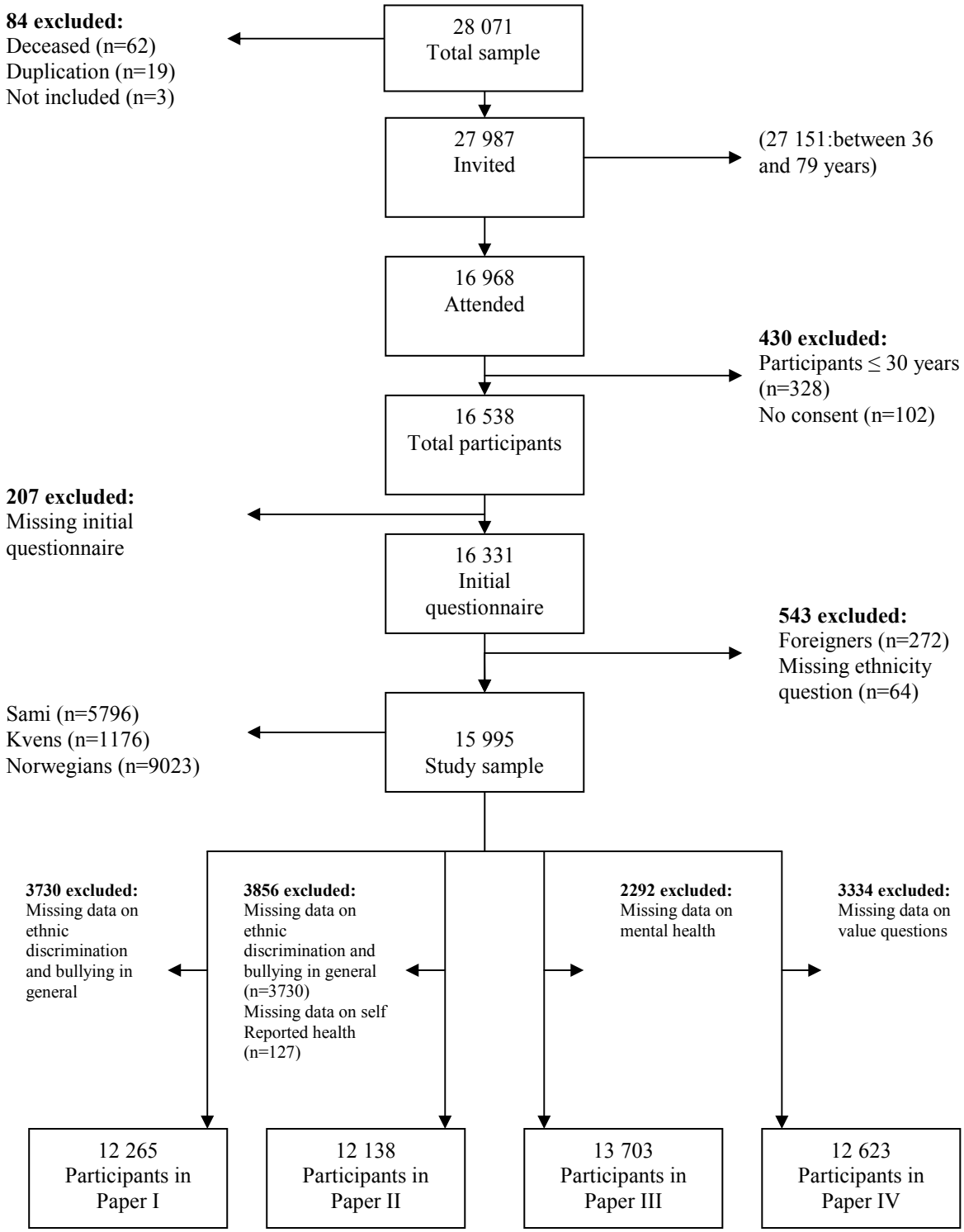
between 1925-1968 and in 1974. This generated an age range in the 2003 partition of 36-78 (plus 30-year-olds born in 1973) and, in the 2004 partition, an age range of 37-79 (plus 30-year-olds born in 1974). However, in analysis, the aforementioned thirty-year-olds were excluded due to low participation numbers (n=328) (Figure 5). Further exclusions were made due to lack of consent to participate in medical research (n=102), missing initial questionnaires (n=207), absence of ethnicity data (n=64) and nationals of other countries (n=272), revealing a study sample of 15,995 (Figure 5). The majority of the sample was represented by people living in rural areas, with municipalities of 3,000 inhabitants or less, with the single exception of Alta, a city with a population of 17,000.

Paper I and II. All participants with complete data on ethnic discrimination and bullying in general were considered: 12,265 individuals aged 36-79 years. Females constituted 51.7%. Ethnic distribution was Sami (33.1%), Kvens (7.8%) and the ethnic Norwegians majority (59.1%). However, in paper II, there were some missing data on self-reported health (n=127).

Paper III. The study sample was restricted to the 13,703 individuals who responded adequately to questions about mental health. In terms of ethnic distribution, the survey recorded 34.6% Sami, 7.9% Kven and 57.5% ethnic Norwegian majority. Females constituted 50.7%.

Paper IV. A total of 12,623 subjects were included in the analysis of personal values. The survey instrument consisted of a 19-item questionnaire and the analysis was based on responses from 10,268 ethnic Norwegian majority and 2,355 Sami participants.

Figure 5: Flow chart for SAMINOR 2003-2004 sample



Questionnaires

The main questionnaire includes five pages of questions, which covered topics such as self-reported disease or illness (including self-reported health status), a check list of mental health symptoms (HSCL-10), ethnicity and linguistic background, socioeconomic factors, bullying in general, use of health services, accidents, diet, smoking, alcohol and physical activity. (Appendix B). After the physical examination, the participants received the additional four-page questionnaire containing more detailed questions of present and past diet with focus on traditional food. In addition, questions focused on social networks, religiousness and 19 questions about personal values, including questions on ethnic discrimination. Thirteen out of the total 19 questions on personal values were specifically connected to aspects of Sami lifestyle and were only presented to Sami participants.

The questions concerning ethnicity were checked in order to avoid omissions and inconsistencies at the physical examination. All questionnaires and the informed consent were available in both Sami and Norwegian languages. The use of Sami language was low: 1.6% responded in Sami to the main questionnaire and 1.3% to the additional questionnaire. In the six municipalities included in the Sami Language Act (Karasjok, Kautokeino, Tana, Nesseby, Porsanger and Kåfjord), the use of Sami questionnaires was higher: 5.7% of the main questionnaires and 4.8% of the additional questionnaires.

MEASURES

Several measures are used in the presented papers. In this section only some essential measures will be presented (ethnicity, ethnic discrimination and bullying, The Administratvei Area of the Sami Language and personal values). The other variables such as socio-economic status, self-reported health, and mental health are described in the respective papers.

Classification of ethnicity

Ethnic identity is the understanding of an individual's affiliation with a certain ethnic group. It can be defined as recognizing which ethic group one belong to by observing the language, tradition, costume, food pattern, norms, values, attitudes and behaviour, and beliefs correspond with those of that specific groups, which are significant for a group and stem from a common original culture transmitted across generations (Bhopal, 2007). In is not easy to accurately depict the ethnic makeup of northern Norway, as the majority of the Sami people live in such close proximity to Norwegians. In addition, Samis do not have a particularly distinct appearance compared to majority-population Norwegians, although there are some known physical characteristics. Also because of forced assimilation, many Sami people today no longer identify themselves as Sami, or don't know about their Sami background, because of assimilation through generations (Høgmo, 1986). And to even make the concept more complex; ethnicity also differ within the same ethnic groups because of differences in age, place of residence, cohort, gender, political association, class, religion, and even personality. Additionally, there are no current demographic numbers to indicate the size of the Sami population due to lack of information on ethnicity in public registers.

We have included a number of questions regarding ethnicity in the survey (Appendix B), as no such data were available in public registers. In the questionnaire the participants were asked about the language that was used at home: for his/her parents, grandparents and self; Sami, Norwegian, Kven or another language (to be specified). For the participants the question on their ethnic background had the same 4 categories of answers, they were also asked about their parents' ethnic background. They were also asked about self-perceived

ethnicity. For all questions, participants were allowed to give more than 1 answer, Based on the above questions, we developed 5 ethnic categories:

1. "Sami I": Maternal and paternal grandparents, both parents and the participant speak Sami language at home.
2. "Sami II": At least 2 Sami speaking grandparents.
3. "Sami III": Sami language or ethnicity for at least one of the grandparents, parents or themselves.
4. "Kvens": Minority of immigrants from Finland.
5. "Ethnic Norwegian": Participants reporting no Sami or Kven affiliation.

Participants with both Sami and Kven background are here considered Sami. Immigrants with other language than Sami, Kven or Norwegian, and born outside Norway are excluded from analyses. Sami I groups correlate very strongly with both self-perceived ethnicity (94.4%) and self-reported ethnicity (97.8%) and feeling of belonging to the Sami culture (Lund et al., 2007). "Sami II" and "Sami III" are more mixed, with both Kven and Norwegian ancestors, and therefore reported weaker relationship to both self-perceived – and self-reported Sami ethnicity. Further details on the categorization of ethnicity in the SAMINOR study have been previously published (Hansen et al., 2008; Lund et al., 2007).

Defining ethnic discrimination and bullying

Discrimination or marginalisation are forms of differential treatment which are not objectively justified and which are not associated with positive discrimination (NOU 2002:12). Ethnic discrimination is often based on stereotypes, that is, unstructured prejudices as derogatory simplifications and generalisations regarding ethnic groups different to one's own. On the other hand, the term 'structural discrimination' refers to systemic social traits that may appear to members of the society as given values, and which have developed over time. Such discrimination may take the form of approved legislation and formal decrees as well as norms and values. Structural discrimination favours specific individuals or groups and rises above individual actions.

However, the term ‘discrimination’ may be defined in different ways. Among the more recent and most exhaustive suggestions we find the definition by Ronald L Craig (Craig, 2007) according to which an action must contain the following four elements to be successfully defined as discrimination:

- an individual or group is in comparison, treated or affected differently than the comparator.
- the difference is disadvantageous to the individual or group.
- the difference in treatment or effect is causally linked to a characteristic of the individual or group protected by antidiscrimination legislation, and
- there is no exception or justification permitting the difference in treatment or effect.

A central feature of this definition is that the ‘attitude’ dimension — the intentions behind individual actions — is considered irrelevant to the evaluation of whether the action is discriminatory or not. This is in line with the *Anti-Discrimination Act* (2005:33).

Questions on ethnic discrimination and bullying

Questions regarding experiences of ethnic discrimination and bullying were asked in two different ways. The first question addressed ethnic discrimination: “Have you ever experienced bullying or discrimination on account of you ethnic background?” The respondents were given the options to range the experiences from “never”, “rarely”, “sometimes” or “very often”. This question was directly related to ethnicity, which meant that the victim was bullied or discriminated against due to her or his ethnicity. The question was not restricted to a time interval in the respondent’s life and is therefore a measure of lifetime experience.

The second question asked about bullying in general. In the questionnaire we have given a short definition about the term bullying to the respondents, which can be translated to English as follows: “With the term bullying we mean repeated exposure over time to negative actions on the part of one or more other person, where the negative actions are through physical contact or verbal abuse, and you are unable to defend yourself against these actions.”

Participants were then asked the question; “Have you ever experienced bullying?” with answering options “Yes, during the last 12 months”, “Yes, previously” and “No”. If the respondents answered “yes” they were then prompted to answer questions about “outcome type” and ”location of bullying”. For types of bullying the respondents had the answering options: “gossiping”, “discriminating remarks”, “being ignored” or “other types”. For locations of bullying the answering options were: “at school”, “at boarding school”, “at work”, “in the local community” or “other places”. For both “outcome type” and “location of bullying” the participants were allowed to give more than one answer. The questions did not give any information as to whether the bullying was due to ethnicity.

The Administrative Area of the Sami Language

In 1990 the Norwegian Government amended the *Sami Act (of 1987) regarding language* to make Sami an official language of Norway specific to the municipalities of Kautokeino, Karasjok, Kåfjord, Nesseby, Porsanger and Tana; today Tysford (2006), Snåsa (2008) and Lavangen (2009) have been incorporated. These municipalities are referred to as *The Administrative Area of the Sami Language* (The Sami act, 1987; Norwegian law, 1997). The purpose of the Act was to safeguard and develop the language, culture and way of life of the Sami people. Within the Administrative Area, the Sami population has the right to receive public correspondence in the Sami language, to use the language in public transactions and adopt the language within the public school system. Even outside these designated municipalities individuals have the right to receive instruction in the Sami language (Jernsletten, 1993).

What are values?

Values may be defined as an individual’s understanding of what is considered to be fundamental goals for one’s own existence and social development (target values, or terminal values) and perceived correct approaches to reach these goals (median values, or instrumental values). This approach to determining values is *descriptive* because it paints a picture of what the members of the population themselves perceive as *the desirable* (Hellevik in Schmidt). A *normative* approach, on the other hand, implies studying what religious, philosophical or other doctrines say about what one should desire; what is desirable. It is also possible to consider

the expectations placed upon the individual by its surroundings, from informal expectations and norms to formal legislation and regulations (Ibid.). The term 'value' is also used in everyday language in a more literal sense about that which is sought after (i.e., a desirable object).

Values have been granted pride of place in many analyses of social conditions. Researchers sometimes use *social background variables* or *characteristics* (such as sex, age, ethnicity, place of residence, level of education, profession and income) to help explain behaviour. Within the social sciences there is a high level of consensus regarding which specific variables are of interest in a survey. Specifically, within research into indigenous peoples, the ethnicity variable is often used to explain differences between the indigenous population and the general population; an example of which may be that Sami people are more concerned with the conservation of ancestral and family traditions than Ethnic Norwegians. One of the questions that spring to mind, then, is what lies behind this ethnological difference? It is not immediately apparent why being Sami or Ethnic Norwegian should be consequential for one's desire to conserve ancestral and family traditions. With such a substantial gap between the presumed *cause* (ethnicity) and *effect* (conservation of ancestral and family traditions) more information about intermediate mechanisms is required to understand what generates the correlation (Ottar Hellevik – *Jakten på den norske lykken*, p. 166).

Attitudes are explanatory variables that are often used to provide insights into such intermediate mechanisms. An attitude is a positive or negative emotional opinion that influences how people act given a certain phenomenon. For example, an individual's or a group's (i.e. the Sami) attitude towards harnessing the wild through fishing, hunting and berry-picking (that is, whether one enjoys or dislikes fishing, hunting, berry-picking) can increase or decrease the probability of 'being in touch with nature'. Should such attitudes be used to explain the importance of 'being in touch with nature' the distance between cause and effect would be so small that the explanation may be taken for granted and the result therefore seems too obvious to be of interest.

However, using attitudes to justify certain phenomena may be difficult because there are so many possible attitudes; perhaps just as many as there are phenomena. Therefore, it would be impossible to create a standardised set of attitude questions in a survey such as the SAMINOR study; on the other hand, social characteristics, which, using a few standard questions and

variations may be used in almost any survey. This is where values become important. As predictor of individuals' behaviour, values are located between social characteristics and attitudes on the influence chain (Figure 6). Values arise from and are influenced by social background and group membership (Sami, Norwegian, Kven). They guide and (may) affect attitudes towards certain given phenomena. Thus, by using values as explanatory variables, some issues relating to social characteristics and attitudes can be avoided. The distance between cause and effect is neither too great to make the findings difficult to understand, nor too small to make them uninspiring. Values, then, can provide meaningful predictions of individual actions without providing, merely, self-evident statements of fact (Hellevik). Furthermore, there are a limited number of values; a standard set of questions on values may be utilised to investigate a wide range of phenomena, as we have done in the SAMINOR study, by developing 19 questions on values.

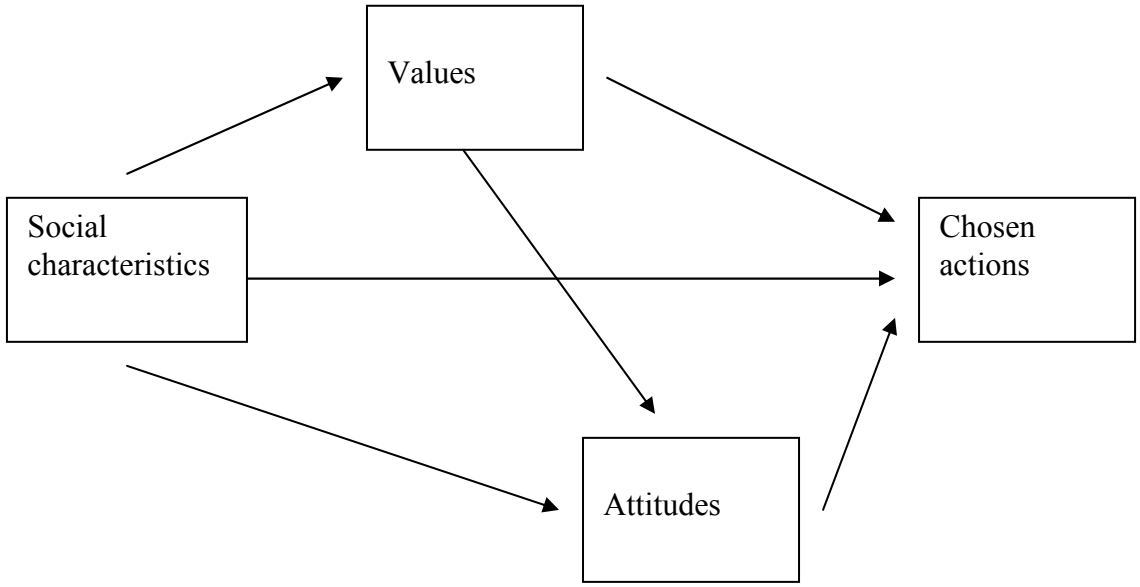


Figure 6: The positioning of values in behaviour-prediction models (after Ottar Hellevik)

How values are connected to ethnic discrimination: 'us' and 'them'

The majority population recognises what is implied by 'Norwegian values': namely, the values that form the basis of Norwegian society. On the other hand, the Sami people assume and emphasise *Sami* values. We know that discrimination actualises implicit and explicit interpretations of whom constitutes 'us' as opposed to 'them'. One may ask, initially, whether

there exist core values that contribute to the separation of the Sami and Ethnic Norwegian peoples. If so, what are these values, and whom wields the power to define these values as definitive, thus labelling ethnic groups holding different values 'dissimilar' and 'divergent'? (Rogstad & Midtbøen, 2009).

In relation to ethnic discrimination, values are of great importance as a mechanism that works to separate those who are on the 'inside' and those who are on the 'outside' of the community. However, due to the development of a multicultural Norway it is no longer possible to equate the 'us', the common language and a shared national heritage with being settled within a nation-state that has clearly defined borders between 'us' and 'them' (Rogstad & Midtbøen, 2009). In connection to the recent Sami revitalisation there has been a need for meaningful Sami symbols and fundamental values, which have been sourced from markedly 'Sami' traditions such as reindeer herding. As a result, many Sami people, particularly inhabitants of coastal regions, have not been identified as 'real' Sami. Thus, because the Sami have been perceived as a homogenous group, many have been excluded from 'the otherness'. Britt Kramvig and Anne Britt Flemmen suggest that diversities and differences have been made invisible in Norway (Berg, Flemmen, & Gullikstad, 2010). This may relate to our finding that Sami individuals living outside the defined Administrative Area of the Sami Language report higher levels of discrimination and poorer health. Individuals of Sami descent struggle with conflicting notions of 'being Sami' due to the number of 'others' in the presumed 'us', assimilation and individuation *as* Sami: all sequences of processes occurring, in some cases, within the lifespan of a single person.

"To be Sami, become Norwegian;
to be Norwegian, become Sami."

(Paine, 2003)

Meanwhile, one must be aware that the Sami people has gained considerable powers to self-rule, which has created the foundations for Sami self-perception that sets aside prescribed identities that demand or suggest particular patterns of living. From the former cultural notions of equality and cohesion grows with renewed force the vision of individual uniqueness, not merely collective uniqueness (Paine, 2003).

Statistical analyses

For statistical tasks, SPSS version 15.0, 16.0 and 17.0 (SPSS INC., Chicago, IL, USA), AMOS for Windows version 7.0 and SAS (version 9.2; SAS Institute Inc., Cary, NC, USA) were utilized. The specific statistical methods employed are described in detail in the individual papers. Statistical significance was reported as p-values.

Ethics

The SAMINOR were carried out in accordance with the Second Helsinki Declaration and were approved by the Regional Board of Research Ethics. In addition, the SAMINOR study was approved bye the Sami consultant at the Board. The National Data Protection Authority (Datatilsynet) gave approval for storing of individuals' information and for later linkages. All participants gave written, informed consent prior to the screening. All participants' related data were anonymized for comparative and statistical purposes.

SUMMARY OF RESULTS

Paper I: *"Ethnic discrimination and bullying in the Sami and non-Sami populations in Norway: The SAMINOR study."*

In Paper I, prevalence of self-reported experiences of ethnic discrimination and bulling was examined. In our survey 1,269 respondents reported having been discriminated against based on ethnicity "Sometimes" or "Very often". This was 10.3 per cent of the respondents in the total sample. Sami and Kven respondents reported significantly more incidents of ethnic discrimination than the majority ethnic Norwegians. Subjects with Sami language in three generations (Sami I) reported the highest prevalence of discrimination. In this group 36% had experience discrimination as happened often or sometimes. Also, in the more mixed Sami groups the reporting of discrimination was high; for subjects with at least two Sami-speaking grandparents the reporting was 19% and among subjects with at least one Sami marks (Sami III) the reporting was 12%. Among Kvens reported 7% that they had experienced ethnic discrimination. Independent of their ethnic group, men reported a significantly higher prevalence of ethnic discrimination and the reporting was highest in the age group 36-57 years. Sami males aged between 36 and 57 belonging to the "Sami I" ethnic group were highly exposed to ethnic discrimination (40.9%). Sami I men living outside the Sami Language Act's district reported the highest prevalence of ethnic discrimination. One other interesting finding is that Sami III, Kven and Ethnic Norwegian reported significant higher prevalence inside the Sami Language Act's district than outside the district (see table 2 below).

Table 2. Prevalence of self-reported ethnic discrimination[a] by ethnic groups, gender and geographical district

		Sami I % (n)	Sami II % (n)	Sami III % (n)	Kvens % (n)	Ethnic Norewgians % (n)
Sami Language Act's district						
- innside the district*						
	Men	35.7 (188)	20.0 (84)	20.9 (28)	12.9 (16)	8.8 (29)
	Women	31.9 (179)	18.0 (76)	13.2 (20)	13.8 (18)	10.1 (44)
- outside the district						
	Men	47.3 (70)	21.4 (109)	12.1 (35)	6.1 (22)	3.0 (92)
	Women	42.7 (53)	15.8 (80)	7.9 (22)	4.4 (15)	2.6 (89)

[a] They who have reported exposure of ethnic discrimination as happened often and sometimes.
* The municipalities: Karasjok, Kautokeino, Nesseby, Porsanger, Tana and Kåfjord.

For bullying in general 403 respondents reported being bullying last year and 2150 respondents previously. Sami respondents reported significantly more bullying, both previously and in the latest year, than the ethnic Norwegians. In the “Sami I” group 37.5% reported being bullied (either previously or last year) compared with 14.9% for the ethnic Norwegians. Overall, Sami respondents reported bullying twice as often than the ethnic Norwegians. Also, Kven respondents reported a higher prevalence of bullying than the majority ethnic Norwegians. Overall, women reported higher prevalence of bullying ($p < 0.001$). For all ethnic groups and both genders the youngest age-groups reported higher prevalence of bullying ($p < 0.01$).

For persons reporting being bullied previously, but not in the latest year, independent of ethnicity, the most common type of bullying was discriminating remarks and the most common locations were public schools. All three Sami groups reported significantly higher than ethnic Norwegians for discriminating remarks ($p < 0.0001$) and that the bullying took place in boarding schools ($p < 0.01$). The ethnic Norwegians compared to the Sami respondents, reported significantly higher levels of gossiping, other types (not specified), and that the bullying took place at work ($p < 0.05$).

For the bullying reported in the latest year, independent of ethnicity, the most common type was gossiping and discriminating remarks and the most common locations were at work and in the local community. For the latest year; discriminating remarks ($p < 0.01$) were reported highest among the Sami respondents. This finding suggests that type and place of bullying were different among Sami and the majority ethnic Norwegians, which suggests that ethnicity has a significant influence on the type of bullying one can be exposed to in Norwegian society.

Paper II: *"Ethnicity, self-reported health, discrimination and socioeconomic status: a study of Sami and non-Sami Norwegian populations."*

Paper II addresses the self-reported health in association with ethnic discrimination, and socioeconomic conditions. Overall, Sami respondents reported inferior health conditions in comparison to ethnic Norwegians. The results show that Sami women living outside the Administrative Sami Area, reported inferior health in comparison to the Norwegian majority population. Also females in general reported poorer health than did males. Health inequalities varied by age and were more apparent in persons aged in their mid-50s or above. Although the disparities between different groups are more prominent in the older age groups, there appear to be no clear-cut trends. For instance, in the age groups 61-65 and 66-70 of the Sami I population higher prevalence of poor health was reported as compared to in the age groups 56-60 and 71-75. Significant associations were demonstrated between poor self-reported health and frequent experience of ethnic discrimination; the odds ratios (95% CI) was found to be 2.88 (1.92-4.32) for women and 1.61 (1.08-2.42) for men, independent of ethnicity. Among the Sami, as well as among the Norwegians and Kvens, those with the highest education and household income indicated better health than others. The paper concludes that ethnic discrimination and low socioeconomic status may explain, at least partly, inequalities in self-reported health between the Sami and the majority population.

Paper III: *"Ethnicity, ethnic discrimination and psychological distress: a study of Sami and non-Sami populations in Norway."*

The objectives of paper III were to (a) examine the prevalence of psychological distress within Sami and non-Sami populations, and (b) to investigate the associations between ethnic discrimination and psychological distress. The results showed that in total, 6.5 per cent of men and ten per cent of women report experiencing psychological distress in the clinical range (measured by HSCL < 1.85). Psychological distress in men was somewhat more prevalent in the Sami groups I and II and in Kvens. In females there were not any significant differences in prevalence of distress between the different ethnic groups. Among those who reported being discriminated against 'Often' and ´Sometimes` the stress levels were higher, 14 per cent in men and 19 per cent in women. Samis who perceived discrimination as happening "Often" and "Sometimes" reported somewhat lower levels of distress than did Kvens and ethnic Norwegian peers — however, this finding was only significant for females ($p < 0.01$).

In general, females reported having psychological problems (for which they have sought treatment) more frequently than males. Sami I females reported less psychological problems than the other females studied ($p < 0.001$).

Ethnic discrimination was strongly associated with psychological distress. Independent of ethnicity, respondents who reported discrimination as happening "Often" were more likely to report distress than those who did not report any discrimination; in females the adjusted estimate was OR = OR = 6.33 (CI 3.88-10.22) and in males OR = 4.50 (CI 2.63-7.69).

Paper IV: *"Sami value patterns"*

The aim of paper IV was to investigate 19 different personal values among Sami and Ethnic Norwegian. Among the Sami the most highly regarded values are: being in touch with nature; harnessing nature through fishing, hunting and berry-picking; preserving ancestral and family traditions; preserving traditional Sami industries and preserving and developing the Sami language. In contrast, Sami respondents' least important values included Sami art and the Sami Parliament (*Sametinget*). In addition, we applied the methods of factor analysis to place values in relation to one another (common variance) and we discover that our questions on values describe four 'value dimensions' within Sami society: Traditional Sami Values, Modern Sami Values, Contact with Nature and Feeling of Marginalisation. Then, we have analysed the dimensions with respect to the characteristics of the different dimensions. The Traditional and Modern Sami Values dimensions were characterised by significantly higher score in females, young respondents and those who had a strong Sami affiliation. In addition, within the Traditional Sami Values dimension, higher scores were recorded in married and cohabiting participants; in those living within the Administrative Area of the Sami Language; those who were satisfied with way of life, and members of the Laestadian Church. As well, within the Modern Sami value dimension, respondents with the greatest household income scored higher. The 'contact with nature' dimension was characterised by significant higher proportions of married or cohabitant subjects and more content with way of life. Finally, participants with high scores on the Feeling of Marginalisation dimension were significantly more likely to be male; in working age; living outside the Administrative Area of the Sami Language; feeling strong Sami affiliation; low household income and more dissatisfied with 'way of life'.

DISCUSSION

To our knowledge, this is the first Norwegian study into ethnic discrimination, bullying and health outcomes in indigenous Sami and non-Sami adults using a large, population-based sample. Research into discrimination and health is growing rapidly and progressing (Williams & Mohammed, 2009). The findings indicate that a large proportion of Sami individuals experience discrimination based on their background, affirming findings from studies into the Sami youth population (Bals, Turi, Skre, & Kvernmo, 2010). Furthermore, our results demonstrate that ethnic discrimination is associated with inferior self-perceived health and psychological distress, which is supported by several other studies across multiple population groups in a wide range of cultural and national contexts (Williams & Mohammed, 2009) including indigenous communities in the circumpolar north (Young & Bjerregaard, 2008). These findings suggest that perceived discrimination is an important emerging risk factor to negative health outcomes.

Ethnic discrimination and the Sami

Although this thesis highlights the situation regarding the Norwegian Sami, it is likely that the issue of discrimination is similar and relevant to circumstances in other Nordic countries as well. The Sami population inhabiting Russia's Kola Peninsula, however, find themselves in a somewhat different situation. Unfortunately, there is less information regarding racism and ethnic discrimination on the Russian part of traditional Sami territory. Consequently, this chapter is limited to Nordic Sami, with particular focus on Norwegian Sami.

The Nordic countries have enacted relatively comprehensive legislation designed to combat ethnic discrimination (Åhren, 2001). Nonetheless the Sami living in the Nordic countries experience prejudices and discrimination both as individuals and as a group. In Sweden the Sami report discrimination in all aspects of society according to the Ombudsman on Ethnic Discrimination (*Diskriminerings-ombudsmannen (DO)*) in a report published in July 2008 (Pikkarainen & Brodin, 2008). Meanwhile, few Sami individuals report such discrimination to the police. The Ombudsman on Ethnic Discrimination Katri Linna states that whilst the Sami have a high level of tolerance for insults and experienced discrimination she encourages the discrimination to be brought to the attention of law enforcement (Labba, 2008). In Finland the Ombudsman for Minorities and the Sami Parliament have repeatedly emphasised the fact that

the Sami have the right to maintain and develop their own language and culture. However, several municipalities fail to implement the right to Sami-language day care as required by the Children's Day Care Act and the Non-Discrimination Act, and not all relevant municipalities provide social welfare and healthcare services in Sami. Problems occur especially outside the Sami homeland (UN Committee on the Elimination of Racial Discrimination (CERD), 2007).

In Norway the Centre Against Ethnic Discrimination (*Senter mot etnisk diskriminering*, or *SMED*) was established in 1998, and in 2006 the country passed a law dedicated to combat discrimination (Ministry of Children, Equality and Social Inclusion, 2005). The purpose of the law was to "promote equality; ensure equal opportunities and rights; and to prevent discrimination based on ethnicity, national origin, descent, skin colour, language, religion and/or worldview". The SMED was abolished in 2005 and its duties transferred on 1 January 2006 to the then recently established Equality and Anti-Discrimination Ombud (*Likestillings- og diskrimineringsombudet, LDO*). Later, in 2009, the Norwegian Government presented a new plan of action to promote equality and prevent ethnic discrimination for the 2009-2012 period (Barne- og likestillingsdepartementet, 2009) as a continuation of the efforts detailed in the former plan against racism and discrimination (2002-2006). The new plan focuses on strengthening the effort against discrimination of the Sami as well as discrimination within the Sami community. As an element of the execution of *Report No. 20 to the Storting (2007-2008): Sami policy* the Government was to take the initiative to invite the Sami Parliament and the Equality and Anti-Discrimination Ombud to a cooperative endeavor against discrimination in the Sami society. The Report refers to attitudes from the *Norwegianisation Policy* lingering in people's minds despite the reversal of policy (Hansen et al., 2008). Thus, it concludes, it is important to maintain focus on questions associated with personal and structural discrimination of the Sami people: "The Government, therefore, will continue to have a strong focus on the discrimination of the Sami (...) Meanwhile, the discrimination and harassment of Sami individuals in the workplace and workforce is a challenge in terms of industrial relations. Continued research into the discrimination of the Sami is important, and to this effect the Centre for Sami Health Research could play an important role" (Report No. 20 to the Storting (2007-2008), Section 4.1.1).

In its report on Norway (2009) the European Commission against Racism and Intolerance (ERCI) advises Norwegian authorities to intensify efforts in several areas. Among other things, it recommends a substantial increase in the availability and application of professional

interpreters in the justice and health systems, and that the implementation of such an increase be prioritised over the two years subsequent to the recommendation. ERCI also advocates incisive research to map out institutionalised ethnic discrimination in the health sector. Finally, the Commission recommends improvements to the monitoring and investigation of racist incidents in general, with a specific awareness of discrimination against the Sami population (European Commission against Racism og Intolerance, ECRI, 2009: Fourth report on Norway)(Hollo, 2009).

On this point, the Government's plan to promote equality and prevent ethnic discrimination (2009-2012) states the following: "It is difficult to determine the population at risk of ethnic discrimination as there are no available data on the numbers of Sami individuals in Norway. For historical reasons there is also significant scepticism among the Sami towards such registration. Hence it is necessary to utilise new methods of illuminating the nature and scope of discrimination. Presently, the collection of such data are distributed across departments, research institutions, volunteer organisations and the Equality and Anti-Discrimination Ombud. There exists a need for a more holistic and systematic collection and synthesis of knowledge ranging from the nature and scope of discrimination in different areas of society to the causes of such discrimination." (*Barne- og likestillingsdepartementet*, 2009).

Sami children and adolescents growing conditions

In the Nordic countries today, Sami youth come of age in a society in which their personal values, culture and language have a completely different status compared to the conditions under which their parents were raised. Young people who are proficient in the Sami language and culture enjoy a more liberated and less politicised definition of "Saminess" than did the preceding generation. Many adolescent Samis express a complex identity including both Norwegian and Sami culture and language; some identify themselves with Kven or Finnish society as well. Among the youngest Sami generation, which families have been exposed to force assimilation. This has cause lots of discussion between the Sami generations, particular when individuals of similar background choose different solutions in order to form their identity. A recent report published by the Nordic Ombudsmen for Children (In Norwegian, *Barneombudene i Norden*, 2008) shows that Sami children are still bullied due to their ethnicity. However, most of them are proud of their Sami identity even though it may be

difficult for some Sami children and adolescents to be honest about their Sami background. Some children are subjected to prejudices within the Sami community, as they sometimes are not accepted as "real Samis" by other Samis.

Health in the Sami population

Overall, previous research on the Sami population presents a uniquely positive situation regarding health compared to indigenous peoples of the Arctic regions of the United States, northern Canada, Greenland, and Arctic Russia. This can largely be attributed to living conditions being largely comparable to those of the non-indigenous populations sharing the same regions (Symon & Wilson, 2009; Young & Bjerregaard, 2008). A possible interpretation of the Sami health situation could be that Sami, Kven and the majority populations in northern Norway have lived side by side in rural multiethnic communities with an almost equally high standard of living (Nystad, 2010), similarities in culture, and equal access to health care services. Indeed, "health status linked to acculturation experiences in a culturally pluralistic society is expected to be better than in culturally monistic one" (Hassler, Kvernmo, Kozlov, 2008). Also, the 'north-south' disparity in Scandinavia is much less marked than that of North America and Russia (Young & Bjerregaard, 2008). This may due to the social democratic hegemony of the Nordic countries.

Meanwhile findings of this dissertation show that Sami and Kven participants report somewhat poorer self-reported health than the Norwegian majority population, and the most unsatisfactory conditions were reported by Sami females living outside the defined Sami areas (Hansen, Melhus, & Lund, 2010). In terms of mental health the findings reveal Sami and Kven males to have higher levels of psychological stress than ethnic Norwegians (Paper III), confirming similar findings from the reindeer-herding Sami males of Sweden (Kaiser, Sjolander, Liljegren, Jacobsson, & Renberg, 2010). Furthermore this dissertation reveals that social factors such as ethnic discrimination may contribute to ethnic inequality in matters of health. Respondents whom reported discrimination were more likely to report adverse self-reported health status and more psychological distress.

Ethnic discrimination and health

Overall, our dissertation suggests that increased levels of perceived discrimination are associated with increased psychological distress and poorer self-reported health status even when income, education, marital status and age are controlled for. These results are consistent with previous research showing that perceived discrimination is associated with a variety of negative physical and mental health consequences (Williams & Mohammed, 2009). However, studies into mental health continue to dominate the discrimination and health literature, and it was precisely between discrimination and psychological distress we found the strongest association. Ethnic discrimination is progressively receiving empirical attention as a class of stressors that may have consequences for health and for understanding disparities in health between minority and majority groups. This is according to the interest in the role of stress as a determinant of social health disparities (Pearlin, Schieman, Fazio, & Meersman, 2005). Psychological stress is associated to and possibly accelerates cellular ageing (Epel, 2009) and "the chronic stressors triggered by multiple environmental assaults can lead to wear and tear on the body that can dysregulate multiple biological systems and lead to premature illness and mortality" (Seeman et al., 2004).

The proper understanding of the relationship between perceived discrimination and health requires a focus on situating discrimination within the context of other health-related aspects of racism; measuring it comprehensively and accurately; assessing its stressful dimensions; and identifying the mechanisms that link discrimination to health (Williams & Mohammed, 2009). However, researchers still disagree on how to conceptualise and measure exposure to ethnicity-related (and socio-economic status-related) stressors (e.g. discrimination and bullying) in accounting for ethnic health disparities over the life course (Myers, 2009). Ethnicity-determined differences in the burden of cumulative vulnerabilities are hypothesised to contribute to differential health status over time. Suggestions are made on the role likely played by ethnicity- and SES-related processes as contributors to persistent ethnic health disparities (Myers, 2009). Research into indigenous peoples worldwide has showed a persistent disparity in health status among many ethnically native groups compared to the respective majority populations (Paradies, 2006). Health outcomes are the by-products of the complex interaction of many factors over time (Myers, 2009). In epidemiological research consensus remains, however, on the major factors that contribute to disease risk although little is known about the complex synergy between the biological, psychosocial, cultural and behavioural explanations which may account for ethnic disparities in health between, for

example, indigenous and non-indigenous populations. “Thus a major public health challenge is to identify the complex set of biopsychosocial factors that contribute to or maintain these persistent health disparities and to design innovative interventions to close the health gaps” (Myers, 2009:9).

Bjerregaard et al. indicate that discrimination and being disrespected could possibly be causally related to high suicide rates and alcohol and drug abuse in many circumpolar communities (Young & Bjerregaard, 2008). Despite the large number of studies investigating the association between discrimination and health, a great number of questions remain unanswered (Pascoe & Smart, 2009).

Design and method

Advantages

- The study was questionnaire-based, and reached a large sample of Sami and non-Sami adults. The influence of random error in the estimated associations between variables was controlled through the inclusion of a sufficiently large representative sample of participants. Therefore, the large sample and the inclusion of different geographic areas make the findings representative, particularly for the Sami adult population of northern Norway.

- Utilising a more strict definition of Sami ethnicity rather than traditional definitions from previous studies, we have had the opportunity to describe some of the heterogeneity within the Sami community. This illuminates the fact that the Sami population has never been a homogenous group in terms of religion, traditions or language.

- Well-established measures of self-reported health and psychological distress were used. This enabled the comparison of Sami and non-Sami populations and enhanced the reliability of findings.

- The study design (questionnaire) is particularly suitable in the research into discrimination, harassment and mental health as such questions may be associated with feelings of shame and guilt and thus could be difficult to answer in an interview format.

Limitations

- There is no consensus in the literature on an optimal measure of exposure to ethnic discrimination and/or harassment. Our study did not address the type of discrimination nor where it had taken place. Questions were not devoted to the identity of the source of discrimination nor whether the discrimination affected the victim. Such information may have been gained through the integration of the interview method.

- To study self-reported measures of discrimination and health is challenging. Reported experiences are subject to recall bias; they are inherently subjective; and, perhaps, not fully captured in a structured questionnaire. Self-reported experiences are unique to the individual and, as such, may not necessarily be representative of the group.

- The cross-sectional design has several weaknesses. It permits identification of important factors associated with discrimination and health and does not allow for conclusions on causality.

- Questionnaire studies are limited. Ideally, interviews or qualitative methods should be added to examine the broader meaning of discrimination, personal values and the affects on well-being and health.

Methodological challenges

Internal validity

Bias can be defined as a systematic error seen when a risk factor or a characteristic applies unequally to comparison groups and this distort the result. It is often classified as selection bias, information bias and confounding (Bhopal, 2008). Biases should always be considered as an alternative explanation of the finding and therefore to be assessed and, if possible, eliminated. A study with high internal validity means that the data collection, analysis and interpretation are correct (Rothman, 2002). With other words, internal validity is the degrees to which the results of a study are correct from the sample of people being studied (Young, 2005).

Selection bias

Selection bias refers to a bias that arises when the study participants are sampled or recruited so that the study sample differs systematically from the population from which it was meant to represent (Brustad, 2004). The attendance rate of 60% in SAMINOR could imply a selection bias if the non-participants had a systematically different prevalence estimate and risk than the participants. Beside the selected areas of our study, the only selection criterion of our study subjects was age 30 and between 36 and 79 years. The non-respondents were more males, subjects in the younger age groups and single. Since, the SAMINOR study was announced as a cardiovascular screening study. The participants may have participated because of a high risk for disease or because they were more health conscious (the "healthy volunteer effect") than those who did not participate (Nystad, 2010). However, a study of non-participants could clarify if selection bias is present. On the other hand, we assume that there is no reason to believe that pattern of perceived discrimination with respect to ethnicity differed between respondents and non-respondents.

Information bias

Information bias may occur if there are systematic measurement errors (continuous variable) or misclassification (categorical variable) of exposure or outcome. Information bias is usually separated into non-differential and differential. Differential misclassification occurs either when misclassification of exposure varies by outcome status or when misclassification of diseases varies by exposure status. This can bias the association on the both directions and can thus be responsible for spurious association. Non-differential information bias usually dilutes the effect of the exposure.

In our thesis, there are two features vulnerable to information bias: measuring perceived discrimination and bullying, and the classifications of ethnicity. Thus, an examination of these features is appropriate.

Measuring perceived discrimination and bullying

The main headline on the front page of the local newspaper *Tromsø* on 24 November 2007 read "Half of the Sami are bullied" with the following comment made by *Sameting* President Egil Olli: "This is frightening!". Several high-profile Sami in Tromsø commented on the SAMINOR figures and said they never had experienced bullying based on their Sami background, although they agreed that such matters are subjectively and individually experienced. Further, it is dependent on what one means by the notion of 'ethnic bullying'. In the article, Nils I. Hætta recalls being yelled at from the stand when playing soccer in the early 1980s. Then, he would be taunted with names such as "goddamn Lapp" (*"jævla lapp"*). However, he did not consider that to be ethnic discrimination, but rather common mockery of the opposing team, which everyone experienced. Further he says that in today's working environment being Sami is an advantage as knowledge of several cultures is perceived positively by employers.

First, this news report illuminates the subjective nature of experienced ethnic discrimination and bullying, and that without having experienced such indignities one may find it difficult to believe that discrimination occurs at all. Social Sciences Professor at the University of Tromsø, Asle Høgmo, deals with this phenomenon in his book, *Stranger In The Norwegian House* (*Fremmed i det norske hus*): "You should not believe that what you haven't seen,

doesn't exist" (Høgmo, 1998). Second, the report highlights the issue of measuring the subjective experience of discrimination. The diagnostic tools of science are analytical notions. Using a metaphor borrowed from medicine: imprecise tools may give imprecise diagnoses, which in turn may lead to improper medication. In relation to research into the Sami people and ethnic discrimination, this means we must strive to create the best possible analytical tools so that we may accurately describe the phenomenon subject to analysis. Thus, it is important to know what we are looking for before we attempt to find it; such that not all unjust behaviour is defined as ethnic discrimination. This is where the real methodological challenges lie.

Consensus is yet to be reached on an optimal measure of ethnic discrimination. Recent studies into perceived discrimination and health have attempted to encapsulate two stressor domains: 'daily hassles' and 'life events'(Williams & Mohammed, 2009). Reliability and validity issues identified in traditional 'life events' scales (recall bias, for example) also apply to most measures of discrimination. Problems relating to recall are more severe in cases in which the recall period is extended; in our particular situation, an entire life span. However, research has revealed that 'severe' events are recalled better than 'less severe' events (Monroe, 2008). Thus, the recall issue may be considered reduced in assessment of the reporting of severe experiences of discrimination. In our study, questions addressed not only ethnic discrimination, but also other types of discrimination. In so doing, the potential phenomenon of perceived unfairness may be recorded more comprehensively. Furthermore, this contributes to reducing the measurement error that may occur when ethnic discrimination, solely, is addressed (Kressin, Raymond, & Manze, 2008). Research into *conducting* research on sensitive matters reveals that underreporting frequently occurs when participants perceive questions regarding discrimination to be socially unacceptable or undesirable (Tourangeau & Yan, 2007). Hence, as researchers, we are at the mercy of respondents, relying on their willingness and ability to recall and report these experiences. To aquire sensitive information, however, self-administered instruments (such as the questionnaire format as used in the SAMINOR study) are preferable to using an interviewer.

Creating classifications of ethnicity

The highly elusive notion — ethnicity — remains, without widely accepted definitions of what it is and how it may be ascertained. Notwithstanding, ethnicity is one of the most significant variables in epidemiology (Bhopal, 2008). "Obviously, before [it] can be used [it needs] to be defined in words that explain, simplify and clarify the underlying complexities and allow the concept to be communicated easily" (Bhopal 2008:30). However, when using ethnicity, one first needs to create population groupings. The process of taking a notion or concept into the realm of practicalities is a pragmatic one. Classifications should be developed based on sound science and logical groupings to the greatest possible extent, with a clear understanding of the purposes of classifying ethnicity. The process involves a considerable amount of subjectivity, and some difficult choices must be made (Bhopal, 2008). Usually the task of creating classifications involves using only a single or a handful of aspects of such complex notions as ethnicity. The accurate application of ethnicity is of utmost importance to improving the health of specific populations (i.e., the Sami population), to reduce inequities and to lay the foundations for good research. What defines someone as Sami has been described in recent acts passed by the Sami Parliaments of Norway (1989), Sweden (1993) and Finland (1995). In all three nations, Sami ethnicity is primarily based on self-identification and secondly on language proficiency, and, even then, direct language skills are not required from the person concerned; so-called retroactive language skills are sufficient. In Russia, by contrast, ethnicity is determined at birth. There, ethnic origin is officially announced by the child's parents (Kulonen, Pulkkinen, & Seurujarvi-Kari, 2005).

In Norway's Sami Act (1987) the definition of 'Sami' extends from a combination of linguistic and subjective criteria. To be recorded in the Sami population registry (in Norwegian: *samemanntallet*) the Act states one must expressly declare that they perceive themselves as Sami *and either* speak Sami at home *or* at least one of whose parents, grandparents or great-grandparents speaks or has spoken Sami at home *or* whom are the descendant of someone already registered in *samemanntallet*. Such a definition may be considered a language-based definition of a Sami population. Ethnic self-identification has been revealed to be an unreliable measure of ethnic background in some studies of the Sami population (Aubert, 1978; Høgmo, 1986); the problem seems to be that some individuals with Sami background are reluctant to report their Sami identity due to stigma. Others, on the other hand, may have some Sami heritage, but consider themselves distanced from Sami *culture*

due to assimilation, intermarriage and having been raised in a predominantly Ethnic Norwegian setting. These are possible weaknesses in looking exclusively at self-identification or family background. However, Sami culture is undergoing a revitalisation process, and the stigma formerly associated with 'being Sami' has subsided or even been reversed. The value in preserving indigenous cultures is increasing in prominence, and some have a strong sense of personal affiliation with Sami values and culture despite loose family ties. Thus, the accumulated and diverse impact in terms of history, lifestyle, assimilation and revitalisation leads to a composite image of the Sami population, in which 'identity' may signify vastly different things to different individuals.

As few non-Sami individuals are proficient in the Sami language, its use within a certain generation may be a reliable indicator of Sami ethnicity. However, the connection remains debatable due to the varying impact of the Norwegianisation process (Aubert 1978:61). Many families have lost their Sami language because of the stigma associated with the process, officially occurring in Norwegian society during 1850-1959. In this period, Sami language was banned from schools and Sami children were required to speak Norwegian only. Thus, particularly in coastal areas (where the Norwegianisation process was especially effective) many Sami people of today do not speak Sami (refer to figure 1 in the chapter "The Sami language") (Jensen, 2005). As a further source of error, survey participants are uncertain about the language spoken by their grandparents. E.g. we have 6% missing data on the language of the participants' grandmothers.

The questionnaire in SAMINOR about ethnic background is asked in three groups. The first group of questions is about the participant, parent and grandparent language. The second group is about the participants and parents ethnic background and the third group is about what the participants consider themselves as (see previous chapter: *'Classification of ethnicity'*)

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For this study (Paper I, II and III), we used the proficiency of Sami Language as the primary marker to categorise the ethnic groups. We are aware that the ethnic classification has limitations, as we have discussed already, since it may have different validity in different geographic regions and within subgroups of the Sami population, However, we chose to use Sami language proficiency to categorise Sami ethnicity because language proficiency has a

high correlation with both self-perceived ethnicity and self-reported ethnicity. Sami I group correlate very strongly with both self-perceived ethnicity (94.4%) and self-reported ethnicity (97.8%) (Lund et al., 2007). Sami II and Sami II are more mixed, with both Kven and Norwegian ancestors, therefore reported weaker relationship to both self-perceived and self-reported Sami ethnicity (see table 3). Thus, they conceive of themselves more as Norwegian than Sami. And therefore it may be more misclassification in the Sami II/III groups. However, this ethnic classification might have led to less misclassification in the SAMI I and Ethnic Norwegian groups (Nystad, 2010).

Table 3: From Lund et al. Populations-based study of health and living conditions in areas with both Sami and Norwegian population – The SAMINOR study. *Int. J. Circumpolar Health, 2007, 66,2; p.123 (58).*

Article IV on ethnic classification centers around the survey question, "What do you consider yourself to be?" — three categories were created based on responses: (i) Sami, (ii) Mixed Sami/Ethnic Norwegian and (iii) Ethnic Norwegian. (Self-identification of ethnicity was used because personal values are closely related to the subjective experience of identity.) In category (i) Sami (n=1,531) 97.1 % consider themselves Sami, 1.5% Sami/Kven and 1.4%

Sami/Other (neither Sami, Kven nor Ethnic Norwegian). In category (ii) Sami/Ethnic Norwegian (n=824) 81.1% consider themselves Sami and Norwegian, 14.1% Sami, Norwegian or Kven, 2.8% Norwegian, Sami and other, and finally 2.1% Norwegian, Sami, Kven and other. In category (iii) Ethnic Norwegian (n=10,268) 97.9% consider themselves Norwegian, 1.4% Norwegian/Kven, 0.7% Norwegian, Kven and other.

Confounding

Confounding is present when a statistically significant association between a risk factor and outcome under study is causally explained by another factor that is also associated to the risk factor under study (Bhopal, 2008). The causal factor is the confounder, and the apparent association between the risk factor and outcome under study is said to be confounded. The confounder can explain all or some of the observed association. It is not always easy to detect confounders. A practical way to achieve this is to analyze the data with and without controlling for the potential confounders. If the estimate of the association differs about 5-10% when controlling for the variable, it is a confounder and should be controlled for in the analyses. Other approaches, based on P-values including in the model variables statistically significantly associated with the dependent variables, are also used. Typical confounders in epidemiology are gender, age and ethnicity. We tried to control for possible confounding through adjustment for age, ethnicity, socio-economic status and other factors in forward stepwise inclusion of the variables in the logistic regression (Paper II and III and IV (linear regression analysis)) and through stratification by sex (Paper I (+age), II and III).We thus have employed strategies to reveal confounding, but we can never know if we have considered all potential confounders.

Sample representativity

The validity of a study refers to whether the findings can be taken as being a reasonable representation of the true situation. A prerequisite for external validity is internal validity. External validity refers to whether the general population in the defined SAMINOR areas systematically differs from the population at large in northern Norway, and whether those who participated in our study (study sample) systematically differed from those not included (Nystad, 2010).

The investigated sample in article I and II (article III and IV has the same demographic characteristics) cannot be considered representative for all Sami in Northern Norway. Only 8.1% (n=998) of the analysed cohort were from Nordland and 7.9% (n=678) from Trøndelag. As well, few of the respondents from Trøndelag reported Sami affiliation (n=49). The majority of the investigated sample was from Finnmark (60%) and Troms (25.1%). Also, it seems less valid for the population in Nordland, due to lower response rate in this area. Furthermore, Sami who live in Southern Norway or urban areas fell outside the scope of the SAMINOR sample. We therefore believe that the results of our study can be generalized to the Sami and non-Sami living in the rural areas of Finnmark, Troms and to some extent also the studied community in Nordland. Nevertheless, at present the sample we used is the best available data source to explore discrimination in a larger Sami population, due to the lack of information about ethnicity in public registers.

Transcultural validation

Cultural insensitivity could arise when researchers uncritically transfer concepts across cultures and develop translations that conform exactly to the original standardized versions without the required adaptations. This kind of approach tends to suppress, bias, and deflect cultural understanding (Vaage, 2010). To ensure semantic, content, and technical equivalence in the SAMINOR study professional translators and bicultural health professionals were involved, together with panel group discussions of the translated instrument, to sort out discrepancies. However, cultural bias could not be completely excluded. For example, as we have discussed in Paper III, lower prevalence of self-reported psychological distress (HSCL-10) among Sami I females than Norwegians females might be due to different attitude towards the diseases and because HSCL-10 is Western-based instrument. A Western instrument alone entails a risk of underestimating the Sami' mental health needs. Vaage et al suggest that it is feasible to integrate universalistic (Western-based instrument) and cultural specific approaches to detect the full range of disabling mental disorders across cultures (Vaage, 2010). Thus, the inclusion of a cultural instrument for Sami adults would have strengthened the methodology of Paper III.

Conducting research into one's own culture

When researching aspects of one's own culture (i.e., the Sami culture) one does not participate in the culture in the usual sense. It is about stepping out of a 'social role' and into a 'researcher role'. Scientists conducting research into their own society or from within their own cultural sphere are more likely to face questions demanding a rationale for one's role in the research (Paulgaard, 1997). As part of her doctorate studies, social anthropologist Vigdis Stordahl (1996) conducted field work in her Sami home town of Karasjok. The reactions she received from academia were on the one hand concerned with the purely personally challenging aspect of doing field work as an anthropologist in her home town (due to anthropological research being perceived negatively in the Sami community). On the other hand, she was warned about the dangers of 'cultural blindness' when doing research "at home". An additional aspect that was raised had to do with 'turning off the Karasjok resident' and 'turning on the researcher' — that is, transforming from 'participant' to 'spectator' (Stordahl, 1996).

The question of whether one may conduct research into one's own culture in an academically defensible manner (being 'on the inside') is a controversial one, particularly within cultural studies. It has been claimed that it is exceedingly difficult to conduct studies within one's own cultural sphere because the researcher lacks the necessary distance to the objects of study (Hastrup, 1991). As a researcher into one's own culture, one brings certain presumptions (latent beliefs) and potentially expect findings based on one's own experiences. However, this does not mean an 'outside' researcher does not have preconceived ideas. To a certain extent, every researcher harbours latent beliefs whether they are 'on the inside' or 'on the outside'. These are merely different types of latent beliefs (Paulgaard, 2000).

Others claim that it is almost impossible to conduct cultural research *without* being 'on the inside'. This statement assumes that it is impossible to understand foreign cultures (Guneriussen, 1996). All in all, the dilemma does not seem to have an immediate resolution. Someone 'on the inside' may have trouble achieving analytical distance; someone 'on the outside' may have difficulty 'getting inside' (as far as cultural understanding is concerned).

As cultural understanding always takes the starting point of the belief or opinion we are already familiar with, the risk of making "ethnocentric mistakes" is greater for researchers 'on the outside' than researchers 'on the inside' (Guneriussen, 1996). Being far removed from the

objects of study in terms of fundamental experience and knowledge may thus be regarded as an obstacle to understanding (Paulgaard, 2000; Rosaldo, 1989). From this point of view it may be argued that cultural proximity (between researchers and their study participants is indeed an advantage for understanding and interpreting research outcomes, including quantitative research. This has, for example, surfaced within Sami political movements where cultural sensitivity has been considered an important competency in research environments (Paulgaard, 2000). In the *Plan for health- and social services to the Sami population in Norway* the point is clearly made that research into Sami issues would be important to compensate for the fact that the Sami as a minority have not been enabled to record their history, or developed scientific approaches and methods. Conducting research into Sami matters on Sami terms would be an important part of making this happen (NOU 1995:6).

The nature of quantitative data collection, on which the SAMINOR study is based, prescribes seeking answers to clearly defined questions posed to participants in questionnaires. For the researcher, this entails indirect contact with the objects of study as opposed to a qualitative approach involving field research. The hypotheses must thus be created prior to data collection. Hence, even within the quantitative tradition, familiarity with the culture one is about to study is advantageous for formulating relevant and culturally specific questions. Further, when conducting research into ethnic groups, the processing of quantitative data requires cultural sensitivity precisely because one does not have direct access to the objects of study.

Conclusions

Main findings

- Sami and Kven respondents reported more ethnic discrimination and bullying in general than Norwegian majority population.
- Males generally reported more ethnic discrimination than females, while females reported more bullying in general.
- Subject with Sami language in three generations (Sami I) living outside the Sami Language Act's district reported the highest prevalence of ethnic discrimination.
- Respondentss with weak Sami affiliation, Kven and Norwegian majority population reported higher prevalence of ethnic discrimination inside the Sami Language Act's district than outside the district.
- For bullying reported in the latest year, discriminating remarks were reported highest among the Sami respondents and the most common locations were at work and in the local community.
- Sami and Kven responders reported poorer self-reported health than the Norwegian majority population.
- Females generally reported having inferior health to males.
- Sami females (especially subject in the Sami I group) living outside the Administrative Sami Area, reported the poorest health. Within the Administrative Area, Kven had the lowest score in terms of health status.
- Ethnic discrimination was associated with poor self-reported health. The findings suggest that ethnic discrimination combined with low socio-economic status contributes to inequalities in self-reported health when Sami and Norwegian majority population are compared.
- Sami and Kven males reported greater levels of psychological distress (HSCL-10) than Ethnic Norwegians. By contrast, Sami and non-Sami females reported similar stress levels.
- Ethnic discrimination was strongly associated with psychological distress.

- Sami males and females with perceived discrimination reported somewhat lower levels of distress than did Kvens and Ethnic Norwegians; however this finding was only significant for Sami females.
- Sami respondents most highly regarded values are: being in touch with nature; harnessing nature through fishing, hunting and berry picking; preserving ancestral and family traditions; preserving traditional Sami industries and preserving and developing the Sami language.
- Four dimensions associated with values were identified: “Traditional Sami Values”, “Modern Sami Values”, “Contact with Nature” and “Feeling of Marginalisation”.
- Feeling of Marginalisation was characterised by: significantly greater proportions of males; feeling themselves as Sami; of working age; living outside the Administrative Area of the Sami Language; low household income; and dissatisfaction with way of life.

Implications for future research

The effort to reduce discrimination against the Sami population primarily revolves around working to inform the populace and to influence generally held attitudes, combined with active policy-making to strengthen Sami values such as the Sami language, Sami schools and traditional Sami industries. The development of Sami institutions represents an important contribution to the visibility of Sami culture. Furthermore, the emphasis on culture and Sami media have been instrumental factors in generating interest and increasing knowledge about Sami matters; Sami culture has become a part of the 'cultural snapshot' of Norway. However, it remains crucial to continue the focus on research into the health and discrimination of the Sami, as pointed out in parliamentary report no. 28 (2007-2008) *Regarding Sami policies* (in Norwegian, St.meld. nr. 28 (2007-2008) *Om samepolitikken*).

To accurately assess exposure to ethnic discrimination one must include the impact of discrimination on others and the intergenerationality of racism. In this regard, the notion of 'historical trauma' if often used, referring to the cumulative psychological distress (in an individual or group) due to a history of genocide or other atrocities inflicted, for example, by European colonisers upon the Sami population (Williams & Mohammed, 2009)). More research is needed to gain further insight into these issues.

Scientific evidence is accumulating in support of the contention that ethnic discrimination leads to adverse health effects. However, additional research is required into useful intervention techniques (on the organisational and individual levels) to identify determinants and reduce the impact and frequency of interpersonal and institutional discrimination. Future population-based inquiries into, specifically, the Sami population are also needed to evaluate the relation between ethnic discrimination and health.

CONTRIBUTION TO THE PAPERS

I: Hansen KL, Melhus M, Høgmo A, Lund E. Ethnic discrimination and bullying in the Sami and non-Sami populations in Norway: The SAMINOR study. *International Journal of Circumpolar Health 2008* Feb;67(1):97-113.

Full Responsibility for the integrity of the study: Hansen. *Study concept*: Hansen, Lund and Høgmo. *Study design:* Hansen, Lund and Høgmo. *Handling ethical and legal aspects:* Hansen. *Analysis and interpretation of data:* Hansen, Melhus, Lund and Høgmo. *Statistical analysis:* Hansen and Melhus. *Drafting of the manuscript:* Hansen. *Critical revision of the manuscript for important intellectual content;* Hansen. *Study supervision:* Lund and Høgmo.

II: Hansen KL, Melhus M, Lund E. Ethnicity, self-reported health, discrimination and socioeconomic status: a study of Sami and non-Sami Norwegian populations. *International Journal of Circumpolar Health 2010*

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IV: Hansen KL, Høgmo A, Melhus M, Lund E. Sami value patterns.

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Paper I

Link to full-text:

http://ijch.fi/issues/671/Hansen.pdf

ORIGINAL ARTICLE

ETHNIC DISCRIMINATION AND BULLYING IN THE SAMI AND NON-SAMI POPULATIONS IN NORWAY: THE SAMINOR STUDY

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ABSTRACT

Objectives. To investigate the prevalence of self-reported experiences of ethnic discrimination and bullying among Sami and non-Sami adults.
Study Design. Cross-sectional, questionnaire-based survey.
Methods. SAMINOR is a population-based study of health and living conditions that was administered in 2003–2004 in 24 different Norwegian and Sami populated municipalities within central and northern Norway. This analysis was based on 12,265 men and women aged 36–79 years. Ethnic distribution was Sami (33.1%), Kvens (7.8%) and the ethnic Norwegian majority (59.1%).
Results. Overall, Sami and Kven respondents reported more ethnic discrimination and bullying in general than ethnic Norwegians ($p<0.001$). The reporting was highest among the younger participants ($p<0.001$). Men reported more ethnic discrimination than women, while women reported more bullying. Respondents with the strongest Sami affiliation reported higher levels of ethnic discrimination outside the Sami Language Act's district, while respondents with weak Sami affiliation, Kvens and ethnic Norwegians, reported higher levels inside this district. Among the respondents that reported bullying previously, the most common type was discriminating remarks and the most common location was public schools. For those who reported bullying in the past year, the most common types were gossiping and discriminating remarks, and the most common locations were at work and in the local community. Two out of three of those reporting ethnic discrimination, independent of ethnicity, also reported bullying.
Conclusions. The findings from this study show that the Sami and Kven population more often experience bullying and ethnic discrimination than ethnic Norwegians. These results are consistent with experiences from other minority and marginalized groups that experienced colonization. More research is needed to understand the role bullying and ethnic discrimination play in the well-being and health of the Sami and Kven population. *(Int J Circumpolar Health 2008; 67(1):97-113)*

Keywords: Ethnic discrimination, bullying, minority, indigenous, Sami, Kven

Paper II

Link to full-text:

http://ijch.fi/show_abstract.php?abstract_id=1013

ORIGINAL ARTICLE

Ethnicity, self-reported health, discrimination and socio-economic status: a study of Sami and non-Sami Norwegian populations

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Received 17 May 2009; Accepted 25 February 2010

ABSTRACT

Objectives. Investigate the association between ethnicity, social factors and self-reported health conditions of Sami and non-Sami Norwegian populations.
Study design. Cross-sectional questionnaire.
Methods. SAMINOR is a population-based study of health and living conditions that was conducted in 24 municipalities in northern Norway during 2003 and 2004. The present study included 12,265 individuals aged between 36 and 79, whose ethnicity was categorized as Sami (33.1%), Kven (7.8%) and Norwegian majority population (59.1%).
Results. Sami respondents reported inferior health conditions in comparison to the Norwegian majority population. The most unsatisfactory conditions were reported by Sami females living outside the defined Sami area (with greater integration and assimilation) ($p<0.05$). Females typically reported less favourable health conditions than did males. Health inequalities varied by age and were more apparent in persons aged in their mid-50s or above. Across ethnic groups, respondents with the highest education and household income were healthier than others. Furthermore, those reporting to have been frequently discriminated against were more likely to report poorer health than those who did not; the odds ratios (95% CI) was found to be 2.88 (1.92-4.32) for women and 1.61 (1.08-2.42) for men. When discrimination was included in the logistical model, the increased risk of poor self-reported health decreased to non-significance for Sami respondents. The estimated risk decreased further when the socio-economic status was taken into account.
Conclusions. The findings of this study suggest that self-reported ethnic discrimination combined with low socio-economic status contributes to inequalities in self-reported health when Sami and Norwegian majority population are compared.
(Int J Circumpolar Health 2010; 69(2):111-128)

Keywords: ethnic discrimination, socio-economic status, self-reported health, ethnicity, indigenous, Sami, SAMINOR

Paper III

Forthcoming:

Transcultural Psychiatry. McGill, Canada

(Accepted 21-Jul-2010)

Ethnicity, ethnic discrimination and psychological distress: a study of Sami and non-Sami populations in Norway

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Abstract

The prevalence of psychological distress and the association between ethnic discrimination and psychological distress was examined among 13,703 participants (36 to 79 years of age) in a population-based study of health and living condition in areas with indigenous Sami, Kven (descendants of Finnish immigrants) and Ethnic Norwegian populations (the SAMINOR study). Sami and Kven males reported greater levels of stress than Ethnic Norwegians. Ethnic discrimination was strongly associated with elevated levels of psychological distress. Given this substantiation, ethnic discrimination is a major potential risk factor regarding mental health which may contribute to ethnicity-related inequalities in mental health between Sami and non-Sami populations.

Keywords: Ethnic discrimination, mental health, psychological distress, Sami, SAMINOR

Paper IV

Forthcoming

SAMI VALUE PATTERNS

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Objectives: To study and contrast personal values in ethnic minority (Sami) and ethnic majority (Ethnic Norwegian) populations in Norway.

Study design: A population-based, cross-sectional study called the SAMINOR study was carried out in 2003-04 in areas of populations with mixed ethnicity.

Method: From 24 municipalities, a total of 12,623 subjects between the ages of 36 and 79 were included in the analysis of personal values. The survey instrument consisted of a 19-item questionnaire and the analysis was based on responses from 10,268 Ethnic Norwegian and 2,355 Sami participants. Associations between personal value variables were assessed using principal component analysis.

Results: From the 19 values, Sami respondents held the following five personal values in the highest regard: being in touch with nature; harnessing nature through fishing, hunting and berry-picking; preserving ancestral and family traditions; preserving traditional Sami industries and preserving and developing the Sami language. On the other hand, Sami respondents' least important values included modern Sami art and the Sami Parliament (*Sametinget*). Four *dimensions associated with values* were identified: "Traditional Sami Values", "Modern Sami Values", "Contact with Nature" and "Feeling of Marginalisation". Traditional and Modern values were both characterised by significantly higher scores among females, the lowest age bracket and those who considered themselves Sami (not including those who considered themselves to be of mixed Sami/Ethnic Norwegian background). Within the Traditional Sami Values dimension, higher scores were also recorded in participants who were married or cohabiting, living within the Administrative Area of the Sami Language, satisfied with 'way of life' and members of the Laestadian Church. The Modern Sami Values dimension was also characterised by higher scores among participants with high household incomes. The Contact with Nature dimension was characterised by significantly higher proportions of Sami (excluding participants of mixed Ethnic Norwegian/Sami background), married or cohabitants, and participants content with their way of life; age, living area and household income was found to be insignificant variables within this dimension. Feeling of Marginalisation was characterised by: significantly greater proportions of males; of working age; living outside the Administrative Area of the Sami Language; considering oneself to be Sami (excluding mixed ethnic background); low household income; and dissatisfaction with way of life.

Conclusion: Four distinct value patterns where identified in the Sami population. The four dimensions reflect important aspects of today's Sami society.

Keywords: Value patterns**,** social indicators, ethnicity, indigenous, Sami, SAMINOR

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